

TRANSFORMING HEALTHCARE

A Five-Pillar Framework with Vermont as the National Proving Ground

ToC - Preface - Introduction - Chapter 1

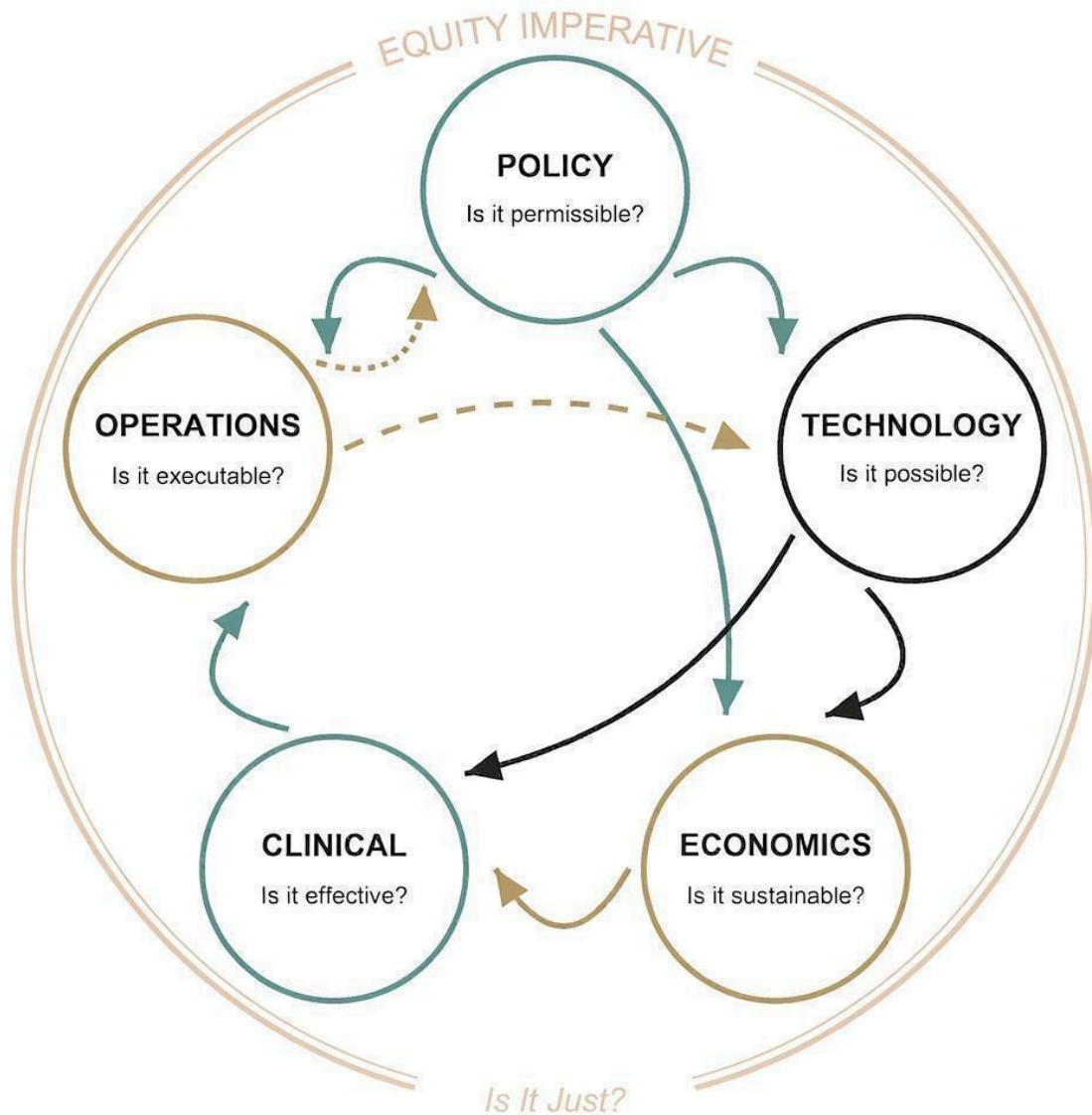


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PREFACE

This book began with a room full of people who already knew something was wrong.

In September 2024, the Green Mountain Care Board (GMCB) of Vermont convened the public meeting — recounted in detail in the Introduction that follows — at which the consulting firm Oliver Wyman presented the findings of a year-long, \$1 million statewide hospital systems analysis. The people in that room had come because they suspected what was about to be confirmed, and because some part of them still hoped they were wrong.

They were not wrong. Nine of Vermont's fourteen hospitals were already operating at a loss. Thirteen of fourteen would be in the red by 2028, absent structural intervention. The cumulative five-year system deficit ranged from \$700 million to \$2.4 billion. Premiums had risen 108% in six years while household income grew 22%. The working-age population that funds the cross-subsidy keeping rural hospitals alive was declining. The over-65 population was growing toward 30% of the state.

That room is the origin of this book. Not because Vermont is uniquely troubled — every American state is facing some version of the same forces on a time delay — but because Vermont is early. Early in its demographic aging. Early in its hospital financial fragility. Early in its recognition that thirty years of voluntary payment reform have not changed the fundamental incentives of the system. And early, therefore, in its willingness to do something that no state has done with this level of statutory specificity and capital commitment: mandate structural change.

BEYOND VERMONT

“On a time delay” is measurable, not rhetorical. In 2020, three states had more residents over 65 than under 18: Maine, Vermont, and Florida. By 2024 there were eleven — Delaware, Hawaii, Montana, New Hampshire, Oregon, Pennsylvania, Rhode Island, and West Virginia had joined them. Vermont now ranks fourth-oldest in the country by median age (42.8), behind Maine (45.1), New Hampshire (43.0), and West Virginia (42.9).

The financial consequence is arriving on the same delay. Nationally, 41% of rural hospitals operate in the red (Chartis Center for Rural Health) and roughly 768 are at risk of closing, 315 of them within three years (Center for Healthcare Quality and Payment Reform). Vermont's nine-of-fourteen — 64% — is not a different problem from the national one. It is the national problem, further along.

This book is the analytical framework for that change. It is not a policy brief for or against any particular agenda. The five goals of Vermont's Act 167 — reduce inefficiencies, lower costs, improve health outcomes, reduce health inequities, and increase access to essential services — are goals that virtually every person across the political spectrum would endorse. The means for achieving them are legitimately contested. This book engages that contestation honestly and presents both sides where the evidence genuinely runs in two directions.

The framework is organized around five pillars: Policy, Technology, Economics, Clinical, and Operations. It emerged from the conviction that transformation fails when it addresses any single pillar in isolation. Payment reform without clinical redesign produces financial savings that evaporate when care models do not change. Clinical quality improvement without payment reform produces better care that the financial system immediately discourages. Technology investment without operational readiness produces platforms that no one uses. Equity goals without structural investment produce aspirations without outcomes.

That last failure is why equity does not appear here as a sixth pillar. It is the Equity Imperative — a single justice test that each of the five pillars must pass on its own terms. The Introduction explains why that distinction is structural rather than a matter of emphasis, and Chapter 1 applies the test pillar by pillar.

The five pillars must move together — which means the people responsible for each pillar must understand all the others.

Vermont runs through this book as its primary teaching case. Vermont is in this book not because it is large or powerful, but because it is transparent. The combination of a small state’s intimacy, a strong public-sector technology foundation in the Agency of Digital Services (ADS) , and an active regulatory body in the legislature, the GMCB, and the Agency of Human Services (AHS) has produced a public documentary record — the multiple legislative acts, the Oliver Wyman analysis, the GMCB’s monthly hospital data, the AHS transformation reports — that gives this book a foundation most states cannot match. Vermont’s transformation is happening in public, with detailed public data, and the lessons transfer.

This book is also not a prediction. The transformations described here are underway but unfinished, and their outcome is genuinely uncertain — the Introduction’s “What This Book Cannot Do” section says plainly what this book does and does not claim to know. What this book provides is the analytical framework for understanding what is happening and why, and for contributing to better decisions as the story unfolds. The decisions, and their consequences, belong to the people who make them.

Finally, this book is not, on its own, the whole of the argument. This book is the intellectual foundation of the **Health Transformation Review**, an open-source integrated ecosystem with three parts designed to work as one:

1. The book (healthtransformationreview.org/book) you are reading — the framework, the dependency logic, the Vermont evidence, and the national comparisons.
2. The platform (healthtransformationreview.org) — five pillar hubs, a Research Lab of nearly forty interactive analytical tools, an AI Analyst grounded in this book and Vermont’s public-domain source documents, a fifty-state dashboard, and a daily intelligence feed. Every argument in these pages that can be modeled *is* modeled there.
3. The Academy (healthtransformationreview.org/academy) — a structured executive-education layer of courses, tracks, and lessons that teach the concepts hands-on and link directly to the tools that apply them.

The three are deliberately cross-linked. Each chapter ends with two doorways: an “*Explore on the Platform*” guide that maps the chapter’s argument to the specific tools that let you manipulate it, and a “*Go Deeper — Academy*” guide pointing to the lessons that teach it hands-on. The platform’s tools carry “*From the Book*” callouts back to the chapters that explain them; and the AI Analyst can answer a question and route you to the right tool or lesson in the same breath. Everything in the ecosystem is, in effect, an interactive expression of this book’s framework. You can read the book alone and it stands on its own — but the framework is meant to be *run*, not just read, and the platform is where you run it.

EXPLORE ON THE PLATFORM

The full ecosystem is at **healthtransformationreview.org** — read the book online with audio narration at /book, open the Research Lab, ask the AI Analyst, or start a course in the Academy. The Introduction that follows explains how the three components fit together and which to start with for your role.

The reader profiles and recommended reading paths are in the Introduction that follows. This material rewards a reading strategy calibrated to your role and your decisions — and to the tools and courses that go with it — not a linear march through the whole book.

December 2025

INTRODUCTION

September 18, 2024

The room was full before the presentation started.

The Green Mountain Care Board of Vermont had convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group would present findings from a year-long, \$1 million statewide hospital systems analysis commissioned under Act 167 of 2022. The analysis had not been produced in isolation: over the preceding year, Oliver Wyman had run more than 230 meetings across all 14 of Vermont's Hospital Service Areas, engaging over 3,100 participants from more than 100 organizations — hospital leaders, state legislators and agency officials, the Vermont Hospital Association, health economists, community advocates, and clinicians. Many of the people who had taken part in that engagement now watched its conclusions arrive. They had come because they already suspected what was about to be confirmed, and because some part of them still hoped they were wrong.

They were not wrong.

Oliver Wyman's lead consultant opened with the financial trajectory. Nine of Vermont's fourteen hospitals were already reporting operating losses, with the worst reaching negative 8.9%. If current trends continued — the same trends that had been building for a decade, the trends that voluntary reform had been addressing for years without reversing — thirteen of the fourteen would be in operating losses by 2028. The cumulative five-year system deficit ranged from \$700 million in the optimistic scenario to \$2.4 billion in the realistic one.

The next slide showed premiums. The average silver exchange plan premium had risen from \$456 per month in 2018 to \$948 in 2024 — a 108% increase. Median household income had grown 22% over the same period. Out-of-pocket maximums had more than doubled. The University of Vermont Medical Center, with approximately 50% of the state's hospital market share, was charging commercial payers roughly 358% of Medicare rates — RAND's published figure for inpatient and outpatient services combined, drawn from 2018–2020 claims — making it, by RAND's own accounting, the most expensive hospital in Vermont. Statewide, Vermont hospitals charge commercial payers 250–300% of Medicare on average, and the Green Mountain Care Board's own later analysis found outpatient imaging at some Vermont hospitals reaching 715–944% of Medicare. Those charges flowed directly into the premiums that Vermont families were paying, with no regulatory brakes.

The final slide showed the demographics. Vermont's working-age population was projected to decline 13% by 2040 — from 367,000 to approximately 318,000 — while the over-65 population rose from 21.7% toward 30% of the state's total. This shift would progressively move patients from commercial insurance, which pays above cost and funds the cross-subsidy that sustains hospitals, into Medicare and Medicaid, both of which reimburse below cost. The slide did not say the words “system collapse.” It did not need to.

Vermont is not an outlier. It is a preview.

No one in that room disputed the numbers. No one argued that the trend would reverse itself. The question that followed — the question this book is an attempt to answer — was whether structural change was possible. Not aspirational change. Not incremental improvement. But actual change to the fundamental incentives, relationships, and capabilities of the system. Whether a small rural state could force a healthcare transformation that the national system, with all its resources and political energy, had been deferring for decades.

Vermont's answer, enacted in Acts 167, 51, and 68 and partially funded by the federal government, is the case study used in this book. Whether it constitutes successful transformation will be determined over the next five to ten years. What is already clear is that Vermont has committed to structural change with more specificity, more statutory force, and more capital investment than any comparably sized system in recent American health policy history.

This book is written in real time, closely watching that transformation. Not as a retrospective account of a completed success, but as an analytical framework for a transformation in progress — with all the uncertainty, potentially incomplete public information, and instructiveness that implies.

What Transformation Actually Means

Few words in healthcare are used more often than “reform” or “transformation,” and the frequency has cost them precision. Both get applied to routine process improvements and to genuine structural change alike, and to aspiration as readily as to commitment — which makes it worth stating, at the outset, exactly what this book means by transformation.

It uses “transformation” in a precise sense: transformation is structural change that alters the fundamental incentives, relationships, and capabilities of a system, producing different behaviors and different outcomes rather than incremental improvements within the existing structure.

By this definition, most of what has been called healthcare transformation over the past fifteen years has not been transformational. Voluntary accountable care organization programs that high-cost providers can decline to join are not transformational. Shared savings models that reward hospitals for reductions in utilization they would have achieved anyway are not transformation. Electronic health record adoption that digitizes paper processes without changing care delivery is not transformation. Quality measurement initiatives that produce reports without affecting payment are not transformation.

Transformation, in the sense this book uses the term, looks like what Vermont is attempting: mandatory reference-based pricing that eliminates the ability of dominant hospitals to charge commercial payers three to four times Medicare rates; statutory global hospital budgets that change every hospital's financial incentive from maximizing volume to managing population health; a Statewide Health Care Delivery Strategic Plan that commits Vermont's Agency of Human Services to specific, measurable outcomes by specific dates; and capital investment through the Rural Health Transformation Program that builds the community infrastructure that payment reform requires to produce population health improvement rather than just cost reduction.

Vermont Is Not an Outlier. It Is a Preview.

Readers from outside Vermont could ask whether Vermont's experience translates to their state, their system, and their community. Vermont is small. Its politics are unusual. Its hospital sector — fourteen nonprofit community hospitals, no investor-owned chains, a regulator with real rate-setting authority — is not typical.

The answer is not found in Vermont's specific characteristics. It is found in the forces driving Vermont's crisis. The demographic aging that is collapsing Vermont's commercial cross-subsidy is present in every Northeastern state now, and will arrive in the rural Midwest, the Mountain West, and the South on a ten-to-twenty-year lag. The failure of voluntary payment reform to reach the tipping point at which fee-for-service becomes the minority mode of payment is a national finding. The administrative burden exhausting physicians and

consuming resources that should go to patients is a national emergency. The pricing opacity that allows dominant hospitals to charge up to four times Medicare rates while rural neighbors close — that is national too.

Vermont faces these forces earlier and more acutely because of its demographics and economic structure. But the same forces are building in every state. The question Vermont is answering — whether mandatory structural reform can actually work — is the question American healthcare will have to answer nationally within a generation.

How to Read This Book If Vermont Isn't Your Context

This book uses Vermont as its primary teaching case for the reasons explained above: transparency, documentation, and an early position on a curve every state is on. But the subject of this book is the five-pillar framework — Policy, Technology, Economics, Clinical, and Operations — and the dependency logic that links them. Vermont is the evidence, not the scope. If Vermont is not your context, here is how to get the most out of the chapters that follow.

Translate the institutions, not just the policies. Every named Vermont statute or institution in this book is a specific instance of a general category that exists, in some form, in every state. The Green Mountain Care Board is Vermont's version of a hospital rate-setting or cost-growth-benchmark authority — Massachusetts, Rhode Island, and several other states have analogous bodies under different names. Acts 167 and 68 are Vermont's versions of a diagnostic mandate and an operational mandate — the categories, not the bill numbers, are what transfer. AHS is Vermont's version of a state health and human services agency acting as a system operator. As you read, ask: what is the equivalent institution, statute, or agreement in my state — even if it is smaller, newer, less formal, or does not yet exist?

Look for the framework statements, not only the Vermont data. Each chapter makes a claim about how the pillars depend on one another that does not require Vermont to be true — for example, that technology investment must precede payment reform because payment reform without data infrastructure cannot be measured or trusted. Vermont's experience is the proof, presented in detail because detail is what makes an argument verifiable rather than asserted. But the claim itself is the point.

Use the “Implications for You” sections as your entry point. Most chapters close by addressing different reader roles directly — hospital executives, payers, state officials, clinicians. These sections are written to apply regardless of any state context, and are often the fastest way to find the actionable content inside a Vermont-heavy chapter. If you read nothing else in a chapter, read that section, then go back for the supporting detail you need.

In short: treat Vermont the way a business textbook treats a case study. The case is specific so that the lesson does not have to be.

Vermont is not the audience for this book. Vermont is the evidence.

The Strongest Case Against This Book's Argument

This book argues that structural, mandatory reform can succeed where voluntary reform has failed. That is a contested claim, and intellectual honesty requires engaging the strongest version of the opposition before asking the reader to proceed on that premise.

1. The Price Regulation Argument

Economists and hospital advocates argue that mandatory reference-based pricing — tying hospital payment to a percentage of Medicare rates — will reduce access rather than improve affordability. If hospitals cannot cover their costs at regulated prices, they will close service lines, exit markets, or reduce Medicaid volumes. Vermont, by this reading, is not pioneering a solution. It is forcing a reckoning that will harm the rural communities it claims to protect.

This is a serious argument. Maryland's experience with hospital global budgets is the primary counterevidence: Maryland has operated all-payer rate regulation since 1977 and maintains rural hospital access comparable to or better than non-regulated states, while holding per-capita cost growth below the national average. But Maryland's model is decades old, built during a different regulatory and demographic environment. Vermont's experiment is newer, its hospitals more financially fragile, and its rural population more dispersed. The access-reduction risk is real and this book does not dismiss it. Chapter 14 (Political Sustainability) analyzes it directly as the primary failure mode to monitor.

2. The Global Budget Argument

Critics of hospital global budgets argue that fixed revenue envelopes create perverse incentives: hospitals that reduce utilization under a global budget model may shift costs rather than eliminate them, delay necessary care to manage within budget, or game quality metrics rather than improve quality. The literature on budget caps and quality outcomes is genuinely mixed.

This book's response is not that global budgets are without risk. It is that the alternative — fee-for-service payment that makes population health management financially irrational — has a documented record of failure that is longer and more extensive than the record of global budget problems. The choice is not between a perfect payment model and a risky one. It is between a payment model that systematically rewards volume and one that, with adequate safeguards, can reward outcomes. Chapter 6 (the Economics Pillar) develops the specific safeguards Vermont's model requires.

3. The Federal Model Reliability and Durability Argument

CMMI has operated innovation models for fifteen years. Most have not produced the savings they projected. AHEAD's predecessor — the Vermont All-Payer ACO Model — produced modest results over nine years despite significant federal and state investment. Why will this time be any different?

This is perhaps the strongest challenge in the book, and the honest answer begins by conceding the premise: CMMI models have generally underperformed their projections, and several have been redesigned or discontinued mid-course. What is different about Vermont's approach is the mandatory statutory architecture its predecessor lacked. Acts 167 and 68 require hospitals to participate in global budgets and reference-based pricing regardless of what happens federally. Vermont's transformation does not depend on AHEAD; AHEAD was to amplify it. This ceased to be hypothetical in July 2026, when Vermont formally withdrew from AHEAD after a CMS renegotiation that cut Vermont's expected EAST Fund from roughly \$138 million to a cap near \$10 million. The mandatory state framework under Acts 167 and 68 remained fully in force, and the reform continued without missing a step. The skeptic's question was answered by events — Vermont did not need AHEAD to survive the loss of AHEAD.

4. The Oliver Wyman Independence Question

This book relies heavily on Oliver Wyman's Act 167 analysis. Oliver Wyman was paid \$1 million by the Vermont state government to produce a report that would support the case for reform. That is not independence. Readers should note that the Oliver Wyman financial projections — hospital deficit trajectories,

administrative cost benchmarks, workforce shortage estimates — cannot all be independently verified from public sources, and the firm had a client relationship that may have shaped how it framed its conclusions.

This book treats Oliver Wyman’s quantitative findings as estimates, not certainties, and notes throughout where specific figures cannot be independently confirmed. The structural argument — that Vermont’s system faces a genuine financial crisis requiring structural response — is supported by public GMCB data, independent KFF analysis, and GMCB’s own enforcement actions that do not depend on Oliver Wyman’s projections.

5. The Vermont-Specificity Argument

The most common version of this objection does not concern any single policy. It is a reaction to the book as a whole: this is a book about one small, unusually homogeneous, unusually transparent state with an unusual regulatory body — does any of it actually apply to a large, diverse, politically fragmented state with a dozen competing health systems and no Green Mountain Care Board?

This objection deserves to be taken seriously rather than answered with a slogan, because the honest answer has two parts, and the second part is a real limitation of this book rather than a rebuttal.

The first part is that the five-pillar framework itself — the dependency logic that determines sequence, the argument that payment reform without clinical redesign evaporates, that technology investment without operational readiness produces unused platforms, that equity gaps require targeted rather than averaged interventions — was not derived from anything unique to Vermont. It is derived from how health systems, payment, technology, and care delivery relate to one another as a matter of structure. A large, fragmented state with a dozen competing systems does not escape this logic; it experiences it in a more fragmented way, with more actors who can defect, more negotiation required to establish anything “mandatory,” and a longer, messier sequence. The logic is the same. The execution is harder.

The second part is the real limitation: this book cannot, in one volume, also provide the equivalent depth of documentary detail for every state’s version of each institution. Vermont’s GMCB, AHS, and AHEAD agreement are described in the detail they are because that detail exists publicly and because Vermont is far enough along its sequence that the consequences of each decision are now visible. A reader in a state without an analog to the GMCB will not find a chapter-length treatment of their own state’s rate-setting politics here. What they will find is a description, in Vermont’s case, of what that function does, why it has to exist in some form for the rest of the sequence to work, and what tends to happen when it does not exist or lacks enforcement authority — which is itself the diagnostic question worth asking about their own state.

Put plainly: a reader who finishes this book having learned a great deal about Vermont but nothing transferable about sequencing, payment design, technology architecture, equity accountability, or operational execution has found a real gap between this book’s intent and its execution — not a correct reading of a book that was only ever about Vermont. The “How to Read This Book If Vermont Isn’t Your Context” section earlier in this Introduction, and the “Implications for You” sections that close most chapters, exist to close that gap.

6. The Transition-Window Argument

Reference-based pricing and global hospital budgets shift a hospital from a system where it can grow revenue by growing volume to one where revenue is fixed or capped. For hospitals already operating below break-even — which, per Oliver Wyman’s own numbers, describes most of Vermont’s system today — the most dangerous financial period may not be the destination (FY2030 statewide global budgets) but the transition itself: the years in which RBP is phasing in, global budgets are not yet operational statewide, and the underlying losses documented in Chapter 6 remain unaddressed. This book documents both the destination and the starting crisis in detail. It is more cautious about the specific financial bridge that carries a hospital across that gap — the combination of Rural Health Transformation Program funds and the sequencing of Medicaid versus

commercial global budgets — which together determine whether a given hospital remains solvent in fiscal years 2027 and 2028 specifically. A hospital CFO's most pressing question is rarely “what does the system look like in 2030?” It is “what keeps us open in 2027?” Chapter 6 (the Economics Pillar) addresses this transition window directly.

7. The Enforcement Question

This book repeatedly emphasizes that Vermont's approach is mandatory rather than voluntary, and treats this as the central correction to OneCare's design flaw. But a mandatory regime is only as real as its enforcement, and a skeptical reader is entitled to ask: what actually happens to a hospital that does not comply — that refuses a global budget, or continues billing above the reference-based-pricing ceiling? Act 68 gives GMCB subpoena authority and data-sharing power with the Department of Financial Regulation, and the answer is no longer hypothetical. GMCB's FY23 enforcement actions against UVMHC (\$80.3M overage) and RRMC (\$11.1M overage) were the first such enforcement actions in Vermont's regulatory history, and UVMHC's subsequent court challenge to that enforcement — Vermont's largest, most powerful hospital system testing GMCB's authority directly — was decided against UVMHC. That outcome is the strongest available evidence that “mandatory” in this book's framework describes a regime that has been tested against its most resourced potential opponent and held. Chapter 13 develops the enforcement mechanics and this case in more detail.

These are not the only critiques of Vermont's approach. They are the most serious ones, and this book engages them throughout rather than assuming the policy case is settled. It is not settled. Vermont is the experiment. This book is the analytical framework for watching it clearly.

The Five-Pillar Framework and the Equity Imperative: An Integrated Architecture

The central analytical contribution of this book is the five-pillar framework for healthcare transformation: Policy, Technology, Economics, Clinical, and Operations. It is not a checklist of things that should be done, but an integrated architecture for understanding why transformation attempts underperform when any pillar is addressed in isolation. When a pillar is missing or disconnected, the transformation cannot deliver its intended result: payment reform without the data infrastructure to manage it, or the clinical models to respond to it, produces financial pressure without improvement.

Each pillar is defined by a single diagnostic question. Policy asks whether a reform is permissible. Technology asks whether it is possible. Economics asks whether it is sustainable. Clinical asks whether it is effective. Operations asks whether it is executable. An honest “no” to any one of them puts the transformation at risk, regardless of how confidently the other four answer yes.

There is a sixth question, and it is deliberately not a sixth pillar: is it just? This is the Equity Imperative — a cross-cutting test that each of the five pillars must pass on its own terms, rather than a separate workstream competing with them for budget, staff, and attention. The distinction is structural, not rhetorical. A sixth pillar could be sequenced last, funded separately, or treated as an adjunct to the work that determines whether disparities actually close. An imperative has none of those escape hatches: each pillar's deliverable carries an equity condition it must satisfy. Does the mandate close disparities or widen them (Policy)? Does the data make disparities visible, or bury them in averages (Technology)? Do the incentives reward serving the

hardest-to-reach populations, or penalize it (Economics)? Effective — but effective for whom (Clinical)? Executable everywhere, including in rural and under-resourced settings (Operations)?

The reason this matters is that a reform can improve the average while widening the gap. Vermont’s primary care access rate is roughly 91% — four points above the national benchmark and a genuine achievement. Yet that aggregate figure conceals an 11-point gap between white Vermont adults and BIPOC Vermont adults (Vermont Department of Health, Health Equity Data Report). The gap becomes visible only when the underlying data is stratified by race and ethnicity. That is why equity cannot be a separate pillar: the five pillars must each be tested for whom they serve and whom they leave behind. A reform that passes every other test can still fail this one.

Chapter 1 develops the five-pillar framework and the dependency relationships among them; Chapter 10 applies the Equity Imperative in depth. What matters here is the premise that connects the book: healthcare transformation is not five separate projects, but an integrated system that must move together — with equity embedded in every pillar from the start.

The HTR Ecosystem: How the Book, the Platform, and the Academy Work Together

This book is one of three integrated components of the **Health Transformation Review** ecosystem. The framework covered in this book is not only an argument to be followed — it is a system to be operated, and the platform (healthtransformationreview.org) and Academy (healthtransformationreview.org/academy) are where you operate it. Understanding how the three fit together will change how you read what follows.

Component	What it is	How it connects to the book
The Book	<i>Transforming Healthcare</i> — the framework, the dependency logic, the Vermont evidence, and the national comparisons. Read it online with audio narration at /book .	The intellectual foundation. Everything else is an interactive expression of it.
The Platform	Five pillar hubs, a Research Lab of nearly forty interactive analytical tools, an AI Analyst grounded in this book and Vermont’s source documents, a fifty-state dashboard, and a daily intelligence feed.	Every argument here that can be modeled <i>is</i> modeled there. Each chapter ends with an “ <i>Explore on the Platform</i> ” guide linking to the exact tools.
The Academy	A structured executive-education layer of courses, tracks, and lessons. Five Pillars, One Imperative is this book’s companion course — eight tracks that follow the book’s own sequence. Start at /academy .	Each chapter ends with a “ <i>Go Deeper — Academy</i> ” guide naming the specific lessons that teach it hands-on.

The connection runs in every direction. The book sends you to the platform through the chapter-ending tool guides. The platform sends you back to the book through “*From the Book*” callouts on every hub and tool. The Academy sends you to both. And the AI Analyst sits across all three — ask it a question and it answers from the book’s evidence base *and* routes you to the tool, dashboard, or lesson that lets you go further. The practical consequence for you as a reader: whenever this book makes a claim you want to test against your own organization or state, there is almost always a tool waiting to let you do exactly that. Every chapter closes with the same two doorways — **EXPLORE ON THE PLATFORM**, then **GO DEEPER — ACADEMY** —

and shorter **TRY THIS** prompts appear mid-chapter wherever an argument is easier to feel in a tool than to follow on the page.

START HERE

New to the ecosystem? Open healthtransformationreview.org/academy/getting-started, declare your role, and the platform personalizes what it surfaces first. Prefer to explore on your own? The Research Lab is organized into benches grouped by pillar — and the AI Analyst can orient you to any of them.

How to Use This Book

This book has four primary reader profiles. Each has a different relationship to the material and a different way of getting maximum value from it — in the book, on the platform, and in the Academy.

The Healthcare Policy Professional

You work in a government agency, a legislative office, a think tank, or a foundation focused on healthcare policy.

Recommended path. Read the Introduction and Chapter 1 (the five-pillar framework and the execution sequence) in full. For the pillar you care most about, read the corresponding chapter in depth. Read Chapter 16 (the AHS Restructuring Roadmap) carefully — it is the most direct bridge between policy mandate and organizational design. Read Chapter 13 (The Future of Healthcare Transformation) for the national context.

On the platform. Start with the HTR Simulator to model a state's readiness and the fifty-state dashboard to benchmark it, then work the Policy & Quality Sciences bench in the Research Lab.

In the Academy. Begin with **Five Pillars, One Imperative**, Track 2 (“Policy — Establish the Mandate”), which works the Act 167 → 51 → 68 cascade and the voluntary-versus-mandatory distinction directly. Then the payment-reform courses at </academy/courses>.

The Healthcare Executive or Administrator

You lead a hospital, health system, health plan, ACO, or healthcare services organization. You are making decisions — about contracts, capital investments, workforce, technology — that the transformation environment is complicating.

Recommended path. Read the Introduction for the overarching argument, then go directly to the chapters most relevant to your immediate decisions. Chapters 6-7 (Economics) for VBC contracts. Chapters 8-9 (Clinical) for care management programs. Chapters 4-5 (Technology) for infrastructure investment. Chapter 11 (Operations) for hospital transformation planning. Return to Chapter 1 for the integrating logic.

On the platform. Run your own organization through the Payment Models & VBC bench — the VBC Readiness Assessment and APM tools — and pressure-test your finances with the Research Lab.

In the Academy. Take **Five Pillars, One Imperative** Track 4 (“Economics — Put Incentives on a Visible System”) for the framework, then the Value-Based Care course at </academy> before your next contract cycle.

The Vermont Practitioner

You work for AHS, GMCB, DVHA, the Vermont legislature, a Vermont hospital, a Blueprint practice, or a Vermont community organization.

Recommended path. Read Appendix A (Vermont System Portrait) first, then Chapter 1 (the five-pillar framework and execution sequence) and Chapter 2 (the Policy Pillar — Acts 167 and 68). Then Chapter 16 (the AHS Restructuring Roadmap) as the operational framework for your work. Use the implementation timeline tables throughout as a working reference guide.

On the platform. The Vermont concept pages — Act 167, Act 68, the AHEAD Model — and the Vermont hospital dashboard turn the book’s Vermont evidence into live, current data. Ask the AI Analyst about any Vermont statute or program.

In the Academy. Five Pillars, One Imperative Track 2 (“Policy”) covers the statutes you work under daily; Track 6 (“Operations — Close the Execution Gap”) covers regionalization and the workforce constraint. The Vermont-specific Medicaid Learning Center is at /academy.

The Healthcare Student or Researcher

You are building foundational knowledge in a graduate program or as an independent researcher.

Recommended path. Read the entire book in order. Chapter 1 establishes the framework and execution sequence; Chapters 2-11 develop each pillar; Chapters 12-16 provide future context and the AHS restructuring roadmap. The Key Concepts sections at the end of each chapter are the glossary. The source notes are the bibliography. Use Vermont as the case study while building transferable analytical skills.

On the platform. The Academy gives you a structured curriculum with progress tracking, and the entire Research Lab is a sandbox for reproducing the book’s analyses yourself. Start at /academy/getting-started.

Profile	On the platform	In the Academy
Policy professional	HTR Simulator to model state readiness; the fifty-state dashboard to benchmark it; the Policy & Quality Sciences bench.	Five Pillars, One Imperative , Track 2 (“Policy — Establish the Mandate”) — the Act 167 → 51 → 68 cascade and the voluntary-versus-mandatory distinction. Then the payment-reform courses.
Executive or administrator	The Payment Models & VBC bench — VBC Readiness Assessment and APM tools — to pressure-test your own finances.	Five Pillars, One Imperative Track 4 (“Economics — Put Incentives on a Visible System”), then the Value-Based Care course before your next contract cycle.
Vermont practitioner	The Vermont concept pages (Act 167, Act 68, the AHEAD Model) and the Vermont hospital dashboard; ask the AI Analyst about any statute or program.	Five Pillars, One Imperative Track 2 (“Policy”) for the statutes you work under daily; Track 6 (“Operations — Close the Execution Gap”) for regionalization and the workforce constraint.
Student or researcher	The full Research Lab as a sandbox for reproducing the book’s analyses yourself.	The Academy as a structured curriculum with progress tracking — start at /academy/getting-started.

Where each profile should start on the platform and in the Academy

What This Book Cannot Do

This book cannot predict whether Vermont's transformation will succeed. The outcome depends on thousands of decisions that will be made by hospital executives, primary care physicians, community health workers, legislators, regulators, and patients over the next five to ten years. This book provides the analytical framework for those decisions; it cannot make them.

This book cannot substitute for the operational expertise required to execute transformation. The frameworks here are accurate and the evidence base is solid, but implementing a global budget requires actuarial expertise, regulatory knowledge, and operational experience that a book cannot provide.

This book cannot resolve the political contests that surround healthcare transformation. Reference-based pricing is opposed by hospital systems that benefit from high prices. Global budgets are feared by hospitals that worry about access restrictions. These are legitimate conflicts of interest that will be resolved by political processes — legislative deliberation, regulatory proceedings, stakeholder negotiations — that this book can inform but cannot replace.

What this book can do: provide the analytical architecture for understanding what is happening and why; supply the evidence base for evaluating what is working and what is not; develop the conceptual vocabulary for communicating across the disciplines — clinical, financial, policy, technological, operational — that transformation requires; and give Vermont practitioners the framework to see their specific work in the context of a coherent transformation agenda. That is enough to have been worth writing — and, I hope, worth your time reading.

Sources: Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024 and January 2025); Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application (November 2025); GMCB VHCURES Data and Analytics documentation; KFF State Health Facts (Vermont); Vermont Department of Health; Act 68 of 2025; Act 167 of 2022; AHEAD State Agreement (January 2025).

Chapter 1: The Five-Pillar Framework and the Execution Sequence

Healthcare transformation is a system problem: five pillars — Policy, Technology, Economics, Clinical, and Operations — connected by nine dependency relationships that determine whether reform succeeds or fails. This chapter opens with the failure that proves the point — OneCare Vermont’s collapse — then defines the five pillars, maps their interdependencies, shows what cascades when any one is missing, and turns to the question those dependencies force: in what order must the pillars be built? Vermont provides both the proof of concept and the canonical failure case — OneCare Vermont’s collapse is the most instructive sequencing autopsy in recent American health policy.

1.1 The Problem with Single-Pillar Thinking

In 2010, the Affordable Care Act created the Center for Medicare and Medicaid Innovation with a mandate to test new payment and delivery models and scale what worked. Over the following fifteen years, CMMI launched more than seventy models, investing billions of dollars and attracting the attention of thousands of healthcare organizations. A 2023 Congressional Budget Office analysis found that the vast majority had not achieved statistically significant reductions in spending or improvements in quality — and that CMMI’s models, taken together, had not saved money at all: they cost \$7.9 billion to operate, reduced spending on health care benefits by \$2.6 billion, and increased net federal spending by \$5.4 billion between 2011 and 2020. Most were discontinued. A handful showed promise and were expanded.

70 CMMI Models Tested (2011–2024)	4 of 70 Expanded Nationwide	15 Yrs Span of Experience	Problem: Single-Pillar Thinking
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The pattern of failure across CMMI models is instructive. Models that addressed only the Economics pillar — changing what Medicare paid without addressing how care was delivered or whether providers had the infrastructure to respond — generated financial pressure without behavioral change. Models that addressed only the Clinical pillar — redesigning care coordination or care management programs — produced clinical improvements that the payment system then penalized by reducing volume-based revenue. Models that required Technology pillar investment — electronic health records, population health platforms, data exchange — found that providers lacked the operational capacity to use the tools effectively.

The lesson is not that payment reform, clinical redesign, or technology investment is wrong. Each one, pursued in isolation, simply runs into the constraints created by the pillars it ignores.

Every pillar you leave out becomes the constraint that defeats the ones you built

Every major healthcare transformation framework in the past thirty years has identified the same list of necessary conditions. Pay differently. Redesign care delivery. Build data infrastructure. Address social determinants. Ensure equity. Execute operationally. The list is right. The reasoning behind it is right. And yet the transformation that the list is supposed to produce has not arrived — not in most states, not at national scale, not with the speed and completeness the evidence has long indicated is possible.

The problem is not the list. The problem is treating it as a list.

A checklist is a set of independent items. You can check them in any order. Checking one does not affect the others. Failing to check one does not prevent you from checking the rest. A checklist of healthcare transformation requirements implies that you can make progress on any one of these independently, and that partial completion is partial progress. This is wrong. The five pillars are not independent items on a checklist. They are a system of interdependent components in which each pillar's effectiveness depends on the others, in which the failure of any single pillar cascades through the system, and in which the order and timing of interventions matters as much as the content of those interventions. Treating the five pillars as a checklist is precisely how decades of technically correct healthcare reform recommendations have produced inadequate results.

1.2 The OneCare Failure: A Sequencing Autopsy

The clearest evidence for that argument is a failure that has already happened. Before defining the five pillars, it is worth examining OneCare Vermont's collapse in structural terms, because the failure was not random and it was not primarily the result of bad intentions or poor execution. OneCare failed because of sequencing errors baked into its design from the beginning.

OneCare was Vermont's primary Accountable Care Organization vehicle under a Total Cost of Care model — holding OneCare financially responsible for the total healthcare spending of an attributed Vermont population. On paper, sound design. In practice, three sequencing failures compounded until the organization could not recover. It wound down after roughly twelve years, founded in 2013 and closing at the end of 2025.

1.2.1 Sequencing Failure 1: Economics Without Policy Foundation

The most fundamental flaw in OneCare Vermont's design was attempting to operate a complex, population-level payment model—requiring system-wide accountability—without a policy foundation capable of enforcing full participation. Vermont lacked the regulatory authority to compel private commercial health plans into the model. This regulatory limit had a specific legal origin. In 2016, the U.S. Supreme Court ruled 6–2 in *Gobeille v. Liberty Mutual Insurance Co.* that ERISA preempts state laws requiring self-insured employer health plans to report claims data into state all-payer databases. That legal limitation became a concrete failure on January 1, 2023, when Blue Cross Blue Shield of Vermont—the state's largest private insurer—exercised its legal right to opt out and refused to renew its OneCare contract. In a single step, Blue Cross removed approximately 93,000 enrollees, stripping away roughly one-third of OneCare's total population base. Blue Cross cited two specific reasons for its exit:

1. Data Governance and Competition: Inadequate contractual safeguards for member claims data under OneCare's plan to subcontract analytics to UVM Health Network, a direct commercial competitor.
2. Lack of Demonstrated Value: No evidence among its enrolled population of measurable progress on cost reduction or health outcomes.

This failure illustrates why the Policy pillar must precede the Economics pillar.

Because federal law limits state authority over self-insured employer plans, state policy cannot rely on voluntary insurer contracts; it must place its mandatory lever directly on hospital rate-setting and global budgets—where state statutory authority remains absolute.

Vermont's Act 68's mandatory architecture is the direct statutory response to this failure. Reference-based pricing and global budgets are not optional — every Vermont hospital is in. But a policy mandate is only the

first of five conditions, and running this one decision through all five is the fastest way to see what the framework does.

WORKED EXAMPLE — ONE DECISION THROUGH THE MODEL

Pillar	Verdict on mandatory hospital global budgets
Policy <i>permissible?</i>	Yes. Act 68 makes global budgets binding on every Vermont hospital — the one lever state law can pull, since federal law cannot compel insurers to participate.
Technology <i>possible?</i>	Not yet. Only if VHCURES and AHS–GMCB analytics can set and monitor each hospital’s budget; without the data, a global budget is a number no one can manage.
Economics <i>sustainable?</i>	Yes, by design. The budget inverts the incentive — every avoided admission now improves margin instead of cutting revenue.
Clinical <i>effective?</i>	Only if delivery follows. The margin is real only when Blueprint care teams actually prevent the admissions the budget rewards.
Operations <i>executable?</i>	Not yet. Running the budgets at all 14 hospitals, not just designing them, requires project-management capacity Vermont is still building.

Vermont’s move to mandatory hospital global budgets (Act 68, FY2028), run through all five pillars. Two clear; three do not — which is the chapter’s argument in miniature.

The Equity Imperative is asked of each of the five, never as a sixth row. Here it lands hardest on Economics: a budget built on historical spending, without social risk adjustment, pays least to the hospitals serving the sickest and poorest Vermonters.

1.2.2 Sequencing Failure 2: Economics Without Technology — The “Managing Blind” Failure Mode

OneCare Vermont’s second structural flaw was attempting to manage a complex, population-level Total Cost of Care model involving real financial risk without the data infrastructure required to execute it.

Managing value-based risk requires two distinct types of data integrated in near-real time:

- **Claims Data:** The financial record of healthcare—every bill submitted to an insurer. It reveals what services were delivered, when, and at what cost.
- **Clinical Data:** The medical record—diagnoses, lab values, medications, and care plans. It explains *why* care was delivered and reflects the patient’s actual health status in real time.

Neither dataset alone is sufficient. Claims data without clinical data creates a financial lag; clinical data without claims data provides no visibility into total spending.

OneCare assumed financial risk without combining these feeds at mature scale. The operational consequences of this gap were documented from two directions:

1. **Internal Recognition:** OneCare’s own leadership acknowledged to Green Mountain Care Board regulators that its analytics infrastructure lagged behind newer systems being adopted by member health networks.

2. Auditor Findings: A state audit revealed the utilization signature of a population managed reactively rather than proactively—patients attributed to OneCare visited the emergency department 36% more frequently than peer ACOs nationally, while recording 18% fewer primary care visits.

This is the “Managing Blind” failure mode: an organization accepts responsibility for what a population costs while seeing that population only in arrears. The worked example below follows a single care manager through both conditions — the lag that makes her work impossible, and the cadence that makes it routine.

Assuming downside financial risk under a Total Cost of Care model without real-time analytics guarantees unexpected financial losses—not because the payment model is flawed, but because the organization cannot see what it is trying to manage.

This failure proves why the Technology pillar must precede the Economics pillar in the execution sequence.

Payment reform without data infrastructure inevitably collapses under unmanaged risk

WORKED EXAMPLE — Why timeliness is as critical as integration

Consider, hypothetically, a care manager responsible for 500 high-risk patients under a Total Cost of Care model who needs to identify patients who are deteriorating before they are hospitalized. If her data arrives two months after the fact, she learns in March that a patient was hospitalized in January. The intervention opportunity is gone. If her data arrives within 24 to 48 hours — integrated clinical and claims feeds updated daily — she can see that a patient filled a new prescription last week, had an abnormal lab result three days ago, and has not seen their primary care provider in six months. She can call today, before the hospitalization. OneCare’s documented utilization pattern — elevated ED use, reduced primary care contact — is consistent with exactly this failure mode, whatever its precise internal lag turned out to be.

1.2.3 Sequencing Failure 3: The Cascade

The two initial sequencing failures did not occur in isolation; they compounded to create a self-reinforcing failure cascade.

Neither failure stayed in its own lane. Losing a third of the attributed population did not merely shrink the denominator — it changed the composition of the very risk pool the analytics were meant to model, so the forecasts degraded exactly when accurate forecasting mattered most. And because OneCare could not see utilization in time to change it, it could not produce the demonstrated value that might have held Blue Cross at the table, or attracted the investment needed to close the data gap in the first place. The policy gap widened the data gap; the data gap foreclosed the evidence that might have closed the policy gap. Each shortfall made the other harder to repair.

These two foundational gaps directly destabilized the Economics pillar leading to contested financial evaluations and conflicting conclusions regarding net savings. Because OneCare could demonstrate neither clear cost savings nor utilization improvements, health systems, payers, and regulators lost confidence and declined to invest further in building the missing capabilities. This skepticism fed back into the cascade, accelerating the model's demise.

Assuming downside financial risk on top of missing policy mandates and missing data infrastructure led to the model's structural decline and its ultimate wind-down at the end of 2025.

Seq. Gap	Structural Error	Direct Consequence	Cascade Effect
Policy (Stage 1)	Payer Mandate Gap: Provider buy-in was achieved (all 14 hospitals), but <i>Gobeille v. Liberty Mutual</i> (2016) blocked state authority to compel self-insured commercial payers into the model.	Payer Opt-Out: BCBSVT exits (Jan. 2023) citing data governance concerns and unproven value—removing ~93,000 enrollees (1/3 of total ACO population) overnight.	Total cost of care cannot be managed or predicted for a fluctuating population → savings targets contested.
Technology (Stage 2)	Data Substrate Gap: Downside financial risk assumed without integrating real-time clinical and claims feeds (analytics lagged member health networks).	“Managing Blind”: Reactive care pattern—auditor-documented 36% higher ED visits and 18% lower primary care visits than national peer ACOs.	Proactive intervention impossible → high-cost acute events not prevented → financial exposure accelerates.
Economics (Stage 3)	Real total-cost risk assumed on top of both gaps above	Independent analyses reach conflicting conclusions on net savings vs. net cost	Contested results → harder to justify further investment in the missing capability
System-level	Compound Failure: Three sequencing gaps compounding over the federal agreement’s ~8-year run.	Model Collapse: Organizational wind-down of OneCare at the close of 2025.	Failure was structural—no management decision could reverse a federal court ruling or rebuild real-time analytics overnight

Figure 1.1 — The OneCare Failure Cascade. Source: HTR Analysis (2026). Based on GMCB records, AHS November 2025 Transformation Report, OneCare Vermont public filings, *Gobeille v. Liberty Mutual Ins. Co.*, 577 U.S. 312 (2016), and VT Digger reporting (Dec. 20, 2022; Mar. 13, 2026).

This complete autopsy illustrates why the execution sequence is non-negotiable in health system transformation:

1. Policy must establish the mandatory statutory architecture to prevent opt-outs.
2. Technology must build the real-time data substrate before financial risk is assumed.
3. Economics can then deploy risk-bearing payment models onto a visible, manageable system.

BEYOND VERMONT

The three sequencing failures above — economics without a policy foundation, payment reform without data infrastructure, and clinical redesign without measurement — are not OneCare-specific defects. They are the three failure modes any value-based care vehicle exhibits when its launch order is wrong, whether that vehicle is a Medicaid ACO in a large fragmented state, a commercial risk contract for an independent physician group, or a CMMI model anywhere in the country. The autopsy is transferable even where the entity, the state, and the payer mix are not.

The next sections build the formal framework this autopsy already implies — the five pillars and the dependencies among them (§1.3–§1.7) — before §1.8 and §1.9 return to state and prove the general rule this case establishes.

1.3 The Five Pillars and the Equity Imperative

Transformation has five moving parts you build and run — and one test they all must pass. The five pillars are capability domains: Policy, Technology, Economics, Clinical, and Operations. Each is a subsystem you can staff, fund, and hold accountable. Each answers a different question about whether transformation will actually work. They are not a row of equals standing side by side. They are load-bearing in sequence: the later pillars rest on the ones built before them, which is why this book treats order as architecture rather than preference.

Equity is different in kind. It is not one more thing you build alongside the five — it is the Equity Imperative every one of them must satisfy. A reform can be permissible, possible, sustainable, effective, and executable and still fail the only test that finally matters: is it just?

Policy — The legislative and regulatory environment that creates the mandatory architecture for transformation. Policy establishes what actors must do, what they are prohibited from doing, and what governance structures ensure accountability. Voluntary reform operates within the Policy pillar's constraints; structural transformation requires it to change.

Vermont application: *Acts 167 and 68 and the Statewide Strategic Plan mandate are Vermont's Policy pillar interventions. Each converts voluntary reform into statutory obligation.*

Technology — The data infrastructure, health information exchange, analytical platforms, and clinical technologies that make population health management possible at scale. The Technology pillar is the information substrate on which most other pillar functions depend — you cannot manage what you cannot measure.

Vermont application: *VHCURES (all-payer claims database), VTTL/VHIE (health information exchange), the AHS-GMCB data integration and analytics capability, AI scribe grants, and Vermont's Clinically Integrated Network are Technology pillar investments.*

Economics — The payment and financial architecture that determines what behaviors the system rewards. The Economics pillar encompasses payment reform (from fee-for-service to value-based and global payment), financial modeling, total cost of care management, and the hospital financial sustainability that transformation requires.

Vermont application: *Reference-based pricing (FY2027), global hospital budgets (FY2028–2030), and total-cost-of-care accountability are Vermont's Economics pillar interventions.*

Clinical — The care delivery models, quality measurement frameworks, clinical workforce design, and patient care programs that determine how care is actually delivered. The Clinical pillar is where payment reform becomes patient outcomes — or fails to.

Vermont application: *The Blueprint for Health (all-payer PCMH program), Collaborative Care Model behavioral health integration, CCBHC expansion, and the primary care investment strategy are Vermont's Clinical pillar interventions.*

Operations — The organizational capacity, management infrastructure, administrative simplification, and implementation discipline that converts strategy into executed programs. The Operations pillar is where transformation plans become real or remain documents — the graveyard of well-designed reforms that could not be implemented.

Vermont application: *The 14-hospital transformation planning process, RHRC technical assistance, the AHS HSA Coordinator model, the Division of Planning and Effectiveness, and EMS regionalization are Vermont's Operations pillar investments.*

Equity — the imperative, not a pillar. The programs, investments, and accountability structures that ensure transformation benefits reach all populations — and especially those most harmed by the current system’s failures. Equity is the test each of the five pillars must pass. A transformation that reduces average costs while widening disparities has failed the Equity Imperative — however permissible, possible, sustainable, effective, and executable it may be.

Vermont application: *geographic equity across Vermont’s 14 Health Service Areas, SDOH investment through Blueprint HRSN screening, rural-access preservation written into the reference-based-pricing and global-budget design, and the health-equity metrics mandated in Acts 167 and 68 are where Vermont holds each pillar to the Equity Imperative.*

The table below specifies each pillar’s diagnostic function and structural role — what it produces when it is working. Equity is not a row here; it is the Equity Imperative applied to every row (see the table above).

Pillar	Diagnostic question	Structural role in the system	What it produces when functioning
Policy	Is it permissible?	The mandatory architecture. Converts aspirational reform into binding requirements. Without mandatory authority, the highest-cost actors opt out and voluntary reform achieves marginal results.	Statutory mandates that eliminate voluntary opt-out; accountability timelines that cannot be deferred; federal-state agreements that commit both parties to specific outcomes.
Technology	Is it possible?	The data substrate. Makes population health management, VBC contract execution, equity measurement, and strategic planning analytically feasible. Without data infrastructure, the other pillars are managed blindly.	Attribution lists, TCOC measurement, HEDIS stratification, risk stratification, care-gap identification, AI-assisted clinical workflows — the analytical functions that VBC and population health management require.
Economics	Is it sustainable?	The incentive architecture. Determines whether organizations have a financial reason to behave differently. Payment reform does not change clinical behavior directly — it changes the financial logic that shapes clinical decisions over time.	Payment models in which population health management is financially rational; price structures that break the commercial cross-subsidy chain; accountability for total cost of care rather than encounter volume.
Clinical	Is it effective?	The mechanism of change. Payment reform changes incentives; clinical redesign changes behavior. The financial case for preventive investment only materializes when clinical programs actually deliver the utilization reduction that produces cost savings and better health outcomes.	Primary care transformation that prevents hospitalizations; behavioral health integration that reduces crisis presentations; care management that closes gaps before they become acute episodes.
Operations	Is it executable?	The execution layer. Translates statutory mandates, payment models, data platforms, and clinical programs into organizational reality. The most analytically sound framework fails if the implementing organization lacks capacity, infrastructure, and management discipline.	Hospital transformation plans that are implemented, not filed; AHS organizational capacity sufficient to manage 14 simultaneous transformation processes; project-management infrastructure that tracks progress against statutory deadlines.

Figure 1.2 — The five pillars: diagnostic questions and structural roles. Sources: Acts 167 (2022) and 68 (2025).

THE EQUITY IMPERATIVE — APPLIED AT EVERY STAGE

There are five justice questions. In the table below, each one is paired with the pillar question it qualifies:

Policy	Permissible — and does the mandate close disparities or widen them?
Technology	Possible — and does the data make disparities visible, or bury them in averages?

Economics	Sustainable — and do the incentives reward serving the hardest-to-reach, or penalize it?
Clinical	Effective — and effective for whom?
Operations	Executable — and executable everywhere, including the rural and under-resourced?

1.4 The Architecture of Interdependency: The Dependency Matrix

The five pillars are linked by nine directed dependencies. Most set the build order; two run backward as feedback once the system is live. Reading those nine relationships accurately is the core analytical skill this book develops, and Figure 1.3 shows all nine at once.

Read each cell as row-pillar → column-pillar: the row sends, the column receives. The verb names the kind of dependency — ENABLES (makes possible), REQUIRES (cannot function without), DRIVES (actively forces change in) — and ∅ marks a feedback loop that runs after the build rather than gating it — Operations INFORMS policy design and RUNS the data infrastructure. There is no Equity row or column, for the same reason the pillar table in §1.3 has none: the justice test applies to every cell. The same map is interactive on the platform at healthtransformationreview.org/about/framework.

FROM ↓ / TO →	Policy	Technology	Economics	Clinical	Operations
Policy	—	ENABLES: funds & authorizes the data build	ENABLES: makes payment reform binding		DRIVES: statutory deadlines force capacity
Technology		—	ENABLES: analytics run the payment model	ENABLES: data enables population health	
Economics			—	DRIVES: incentives reshape care delivery	
Clinical				—	REQUIRES: care models need execution infra
Operations	INFORMS ∅: data refines the mandate	RUNS ∅: workforce runs the infrastructure			—

Figure 1.3 — The Dependency Matrix. Read each cell as row-pillar → column-pillar. Sources: Acts 167 (2022) and 68 (2025).

Where two pillars depend on each other, the two arrows carry different content — Policy → Operations is a deadline, Operations → Policy is implementation data — so each is a genuine relationship, not one fact counted twice.

What Earns a Place in the Matrix

Every cell in Figure 1.3 has to justify itself, and so does every empty one. A grid built from relationships that merely struck someone as important cannot answer the question a sceptical reader should ask: why is this cell filled and that one empty? So the matrix follows a single rule.

The rule is narrow on purpose: a dependency earns a cell when the downstream pillar **cannot** produce its **intended** result until the upstream pillar delivers something specific and **nameable**. **Cannot** means the downstream pillar is genuinely stuck without the upstream one — not just better off. “Benefits from” and “works better with” don’t qualify. **Intended result** keeps the test honest: what’s at stake has to be the downstream pillar’s own output, not a general sense that the upstream pillar helps. **Nameable** is the practical check: you have to be able to say exactly what gets handed from one pillar to the other. If you can’t name it, what you’ve found is a resemblance between two pillars, not a real dependency.

Naming what flows is easier than it sounds, because each pillar issues exactly one thing: its currency. That is what makes the five pillars distinct rather than five labels for “important stuff.” The table below sets the five currencies side by side.

Pillar	What it issues (currency) — and what that means
Policy	AUTHORITY — Mandate, prohibition, statutory deadline — and the power to appropriate. Anything that makes an action required rather than advisable.
Technology	INFORMATION — Data that can be trusted, at a cadence fast enough to act on. Claims, clinical feeds, risk scores, stratification.
Economics	INCENTIVES — The financial consequence attached to a behaviour — what the system rewards and penalises. Reference-based pricing, global budgets, total-cost-of-care accountability.
Clinical	OUTCOMES — Care actually delivered, and the measured result of delivering it. The only currency a patient experiences directly.
Operations	CAPACITY — People, credentials, management infrastructure and project discipline — the ability to run what the other four design.

What each pillar issues. A dependency exists only where one pillar needs what another pillar issues.

The test follows directly. Name what flows. Ask which pillar issues that currency. If it is the upstream pillar, the cell is real. If it is a different pillar, the relationship belongs in another cell — or it is one already in the grid, read from the wrong end.

Two mistakes are easy to make.

The first is counting one relationship twice. Technology → Economics is a single relationship: Technology issues information, and Economics needs it. To say instead that “Economics depends on Technology” describes the same relationship from the receiving end; it is not a second cell. The matrix records each relationship once, from the pillar that issues.

The second is confusing one currency for another, and it is the harder mistake to avoid because the intuition behind it is sound. Data infrastructure is expensive; VHCURES, a health information exchange and an analytics capability cannot be built without money; and money seems like Economics. So the grid ought to contain an Economics → Technology cell. It does not, and tracing the money shows why.

Vermont's technology build is partially funded by the Rural Health Transformation Program — a \$195 million first-year award — and was to have been supplemented by the EAST Fund under the AHEAD State Agreement, until Vermont withdrew from AHEAD in July 2026. Both are appropriations: money made available by statute or federal agreement, for named purposes, on a schedule someone else set. An appropriation is an exercise of authority, and authority is what Policy issues. That is why the Policy → Technology relationship is described in this chapter as funding and authorizing the build: the two arrive together because they are one instrument.

What the Economics pillar issues is consequence, not capital. Reference-based pricing, global budgets and total-cost-of-care accountability change what a hospital gains or loses by behaving one way rather than another; none of them pays for a server. A state can have a complete incentive architecture and no data infrastructure at all — which is close to what Vermont had under OneCare, and §1.2.2 is the account of what that cost.

The distinction between transformation capital and incentive architecture is not a bookkeeping point. Bridge funding is time-limited and ends; incentive architecture is permanent and compounds. A program built on capital and mistaken for one built on incentives has an expiry date nobody has written down. Vermont's AHEAD withdrawal shows it: the EAST Fund was cut from about \$138 million to a cap near \$10 million, and the incentive architecture — Act 68's mandates — was untouched, because the two were never the same thing.

So there is no Economics → Technology cell. The relationship is real, but what it proposes to send has a different sender. Applied honestly, the rule will sometimes delete a cell you are fond of; that is what a rule is for. A reader who disagrees with any of the nine relationships now has something specific to argue with: name the currency, and name its issuer.

WHAT A “GATE” MEANS IN THIS BOOK

Picture the pillars as a series of locked gates on a single road. A pillar's gate is the work that pillar must complete before traffic can pass through to the pillar behind it.

A gate is OPEN when that pillar's prerequisite work is finished — the statute is enacted, the analytics are running, the contracts are signed. Downstream investment can now proceed and produce its intended value.

A gate is CLOSED when that work is unfinished. Money spent downstream of a closed gate does not fail loudly; it fails quietly, producing results nobody can use until the gate opens. That is why the sequence cannot be bought past — the point Principle 1 develops later in this chapter.

You open a gate by completing the work, not by deciding to proceed. A gate opens on evidence: the capability exists and is operating. Declaring a milestone met does not open it.

Throughout this chapter, “open” always means *passable* — *the work is done*, and “closed” always means *blocked* — *the work is not done yet*.

The sections that follow take the nine relationships one source pillar at a time: the mechanism, the failure mode when it is ignored, and the Vermont evidence that confirms it. Each section closes with the same question — whether that pillar passes the Equity Imperative.

1.4.1 Policy Dependencies: Three Relationships That Establish the Mandatory Architecture

Policy is the pillar from which mandatory authority flows. Its outbound dependencies — to Technology, Economics and Operations — are among the most critical in the framework, because they are the relationships that convert every other pillar from optional to required.

Policy → Technology: statutory funding and mandate authorize the data build · *ENABLES*

The data infrastructure a transformation runs on does not get built because it is a good idea. Act 68 and the Rural Health Transformation Program fund and authorize Vermont's build — VHCURES, the AHS–GMCB analytics capability, the health information exchange. Without statutory funding and a statutory deadline, the Technology pillar has no forcing function, and it is the pillar most often deferred: it is expensive, it produces nothing visible to patients in its first years, and every other pillar can be started without it. That is precisely why Policy has to open this gate first. OneCare assumed financial risk on analytics no one had been compelled to build, and §1.2.2 is what that cost.

Policy → Economics: Mandatory authority enables structural payment reform · *ENABLES (critical path)*

Payment reform that is voluntary produces voluntary results. OneCare Vermont ran for twelve years on a voluntary model: all 14 hospitals took part, but the insurers did not have to — and when Blue Cross Blue Shield of Vermont, the state's largest, left in January 2023, about a third of the attributed population went with it (§1.2.1). Act 68 of 2025 changed the architecture where the state does have authority: reference-based pricing is mandatory for all Vermont hospitals beginning FY2027, and global budgets are mandatory beginning FY2028. That closes the hospital-side opt-out. The payer-side limit set by federal law remains.

VERMONT EVIDENCE

Oliver Wyman's Act 167 diagnosis explicitly found that voluntary reform was insufficient given the financial trajectory. The \$700 million to \$2.4 billion five-year deficit projection assumes the continuation of the commercial pricing structure that mandatory RBP is designed to dismantle. Without the Policy pillar's mandatory authority, the Economics pillar remains aspirational.

BEYOND VERMONT

Maryland has been running the experiment Vermont is starting. Its all-payer model put essentially every hospital in the state — 52 of them — under global budgets beginning in 2014, mandatorily, with no opt-out. A decade of evaluation is the closest thing American health policy has to a controlled test of this dependency.

The results support the mechanism and complicate the optimism. Medicare spending fell 2.8% against a comparison group, driven by a 4.1% reduction in hospital expenditures — more than \$780 million saved. Potentially preventable admissions fell 16.1%, unplanned readmissions 9.5%. But the quality picture was mixed: readmissions improved for healthier patients while hospitals struggled with sicker ones, and population health measures did not move.

The lesson that transfers is narrower than “global budgets work.” It is that mandatory architecture reliably changes hospital financial behavior — which is what this dependency claims — and that changing financial behavior is not the same as improving population health. That second step requires the Clinical pillar, and Maryland's experience is the evidence that it does not arrive on its own.

Policy → Operations: Statutory deadlines force execution capacity · *DRIVES*

Organizations develop the capacity to execute complex initiatives when they have non-negotiable deadlines, not when they have good intentions. Act 68's December 2028 Statewide Strategic Plan deadline, the FY2027 RBP mandate, and the FY2028 global budget mandate are not just policy milestones — they are organizational forcing functions that require AHS to build the project-management infrastructure, analytics capability, and staffing depth that the transformation requires. Without the Policy pillar imposing external accountability, the

Operations pillar faces the chronic problem of healthcare administration: transformation planning that is perpetually in progress but never complete. Vermont's statutory deadlines are the mechanism that prevents this.

The Equity Imperative on Policy — does the mandate close disparities or widen them?

Acts 167 and 68 write equity into statute as operating requirements, not aspirations: equity metrics must be tracked, disparities must be reported, and the Statewide Strategic Plan must address geographic and demographic equity gaps. Without statutory embedding, equity monitoring is a discretionary activity that competes with operational priorities; with it, equity measurement is a compliance requirement. The test also applies to how each instrument is designed. A reference-based-pricing methodology that sets rates uniformly, without adjusting for the market conditions of hospitals in underserved communities, is technically correct and practically inequitable — which is why Vermont's policy decisions, from the RBP rate methodology to the Statewide Strategic Plan, have to be evaluated for equity impact before they are finalized, not after.

VERMONT EVIDENCE

The Health Care Delivery Advisory Committee established under Act 68 is required to consult with the Health Equity Advisory Commission and to incorporate equity considerations into the Statewide Strategic Plan. This institutional design is the Policy pillar building the equity imperative's accountability structure into the statutory framework.

1.4.2 Technology Dependencies: Two Relationships That Enable Analytical Management

Technology is the data substrate on which every other pillar's management functions depend. Its two outbound dependencies, to Economics and to Clinical, describe what becomes possible when data infrastructure is adequate and what stays impossible when it is not.

Technology → Economics: Analytics makes VBC financial management possible · *ENABLES (critical path)*

This is the relationship the matrix records once, from Technology's side: Technology issues information, and Economics cannot manage a payment model without it. The difference it marks is between a contract existing and a contract being manageable. An organization can enter an APM contract without adequate analytics — the contract still exists, the financial obligations are still real. But it cannot manage its financial performance under that contract, cannot identify where its costs are above benchmark, cannot target care management toward the highest-return opportunities, and cannot dispute errors in the payer's shared-savings calculations.

WORKED EXAMPLE — Signing a contract you cannot read

A CFO signs a two-sided-risk contract for an attributed population of 12,000 lives, with a total-cost-of-care benchmark set from three years of historical claims. Every clinical decision her system makes over the following year is defensible. The care management team is good. The physicians are engaged. In month fourteen, the payer's reconciliation arrives: her system is \$2.1 million over benchmark and owes a share of it back.

She cannot tell whether that number is real. To dispute it she would need to know which patients were attributed and when they moved in or out of the panel; what the risk scores were and whether her documentation supported them; which spending happened outside her system, where she had no visibility at all; and whether the benchmark's trend factor matched what actually happened in her market. She has none of it. The payer has all of it. So she pays. Not because her system performed badly — she still does not know whether it did — but because she entered an argument she had no evidence to make. The clinical work was never the problem. The Technology gate was closed before she signed, and signing did not open it.

VERMONT EVIDENCE

Act 68 requires GMCB, beginning in FY2028, to set prospective annual hospital global budgets based on historical spending adjusted for population risk. That calculation needs accurate VHCURES data, a validated risk-adjustment methodology, and an analytics platform able to produce hospital-specific total-cost-of-care projections. The AHS–GMCB data integration and analytics capability being stood up in 2025–2026 is the Technology investment that makes global-budget implementation executable. The same VHCURES logic underlies the APM Shared Savings Calculator, the Hospital Financial Stress Test and the VBC Readiness Assessment in the HTR Research Lab (Chapter 7).

BEYOND VERMONT

This dependency is where most states are structurally disqualified before they begin. Roughly two dozen states operate an established all-payer claims database (APCD Council); the other half of the country does not have one. Vermont's VHCURES is not an unusual advantage in kind — it is an unusual advantage in *timing*, built years before the payment model that now needs it.

For a state without an APCD, the sequencing consequence is concrete: total-cost-of-care accountability cannot be measured, so a global budget cannot be set defensibly, so the Economics pillar cannot be operated even where the political will exists to mandate it. The gap is not a year of procurement. Standing up an APCD is multi-year work involving legislation, payer reporting mandates, and data-governance infrastructure.

Read against the Policy → Economics box above, this is the practical test for any state reading Vermont's playbook: mandatory authority is necessary but it is not sufficient, and the Technology gate is the one that cannot be opened by a vote.

Technology → Clinical: Data infrastructure enables population health management · *ENABLES*

The clinical programs developed later in this book — Blueprint PCMH, CoCM, CCBHC, CHT deployment — can all be designed and implemented without data infrastructure. What they cannot do without it is function as population health programs rather than individual encounter programs. Risk stratification identifies which patients are most likely to deteriorate; without it, care management deploys reactively, after hospitalization rather than before. Care-gap identification triggers outreach for overdue preventive services; without it, gaps close only when patients schedule appointments. SDOH screening tools flag patients for CHT navigation; without the data platform to connect the flag to the navigation workflow, the flag is generated and ignored.

VERMONT EVIDENCE

Blueprint's clinical registry is the Technology pillar product that enables Blueprint's Clinical pillar outcomes. Blueprint's 5.8:1 ROI, taken up under Economics below, rests here on a narrower condition: the registry is what lets a care team work a population rather than an appointment list. The AI scribe grants in the RHT Program are the Technology pillar's contribution to the Clinical pillar's workforce capacity problem.

The Equity Imperative on Technology — does the data make disparities visible, or bury them in averages?

The equity imperative depends on the Technology pillar's ability to produce demographic stratification. You cannot measure a disparity you cannot see. The 91% primary care access rate and the 11-point white/BIPOC gap behind it — both set out in the Introduction — are the same population counted twice: once in aggregate,

once stratified. Only the second count shows the gap — and VHCURES can produce that count only when the underlying data carries adequate race and ethnicity detail.

VERMONT EVIDENCE

Vermont's race/ethnicity data in VHCURES is incomplete — commercial claims frequently lack demographic fields. This is not a minor data quality issue; it is a Technology pillar gap that directly limits the equity imperative's analytical capability. The Vermont Department of Health's Health Equity Data Report (January 2025) documents this limitation explicitly. Closing the Technology gap is prerequisite to the equity imperative's disparity measurement function.

1.4.3 Economics Dependencies: One Relationship That Transmits Incentive Change

Economics is the pillar through which policy mandates are converted into behavioral change at the organizational and clinical level. That conversion is always indirect, which is why its one outbound dependency — to Clinical — matters more than its own design.

Economics → Clinical: Payment incentives drive care delivery redesign · *DRIVES (critical path)*

This is a critical dependency. Under fee-for-service, preventing a hospitalization costs the hospital revenue. A primary care practice that successfully manages a diabetic patient's condition and prevents a \$15,000 hospitalization has generated no billable encounter for the prevented admission. The clinical logic and the financial logic point in opposite directions. Under global budgets, the calculus reverses: every avoidable hospitalization is a cost against a fixed revenue envelope, and every prevented admission is a margin improvement. The payment model makes population health management financially rational for the first time.

VERMONT EVIDENCE

Vermont's Blueprint for Health demonstrates this dependency in reverse. The Blueprint's 5.8:1 return on investment — \$5.8 million in reduced medical expenditure per \$1 million invested in primary care — is achievable because the Blueprint's all-payer PMPM payments to PCMHs create a financial structure that rewards the clinical investment. Sustain the Economics pillar through Act 68's global budgets and the Clinical pillar's population health logic becomes fully incentive-aligned.

The Equity Imperative on Economics — do the incentives reward serving the hardest-to-reach, or penalize it?

Value-based care contracts that do not adjust for social risk systematically penalize providers serving high-SDOH populations. A hospital in Essex County, Vermont — serving a population with higher poverty rates, housing instability, food insecurity, and transportation barriers — faces higher care management costs and lower quality measure performance potential than a hospital in Burlington serving a more economically stable population. If their global budgets are set at the same percentage of Medicare rates without social risk adjustment, Essex County's hospital is being held to a standard that does not reflect its population's actual cost burden.

VERMONT EVIDENCE

Vermont's hospital global budget design under Act 68 must address this explicitly. The methodology for social risk adjustment in Vermont's hospital global budgets is an active design question as of early 2026. Getting it wrong — designing an Economics pillar instrument that ignores the equity imperative's requirement — would produce a payment system that penalizes the hospitals serving the most vulnerable Vermonters.

1.4.4 Clinical Dependencies: One Relationship That Connects Care Models to System Requirements

Clinical is the pillar that converts payment incentives into actual health outcomes. Its one outbound dependency — to Operations — describes what a care model needs in order to be delivered at all.

Clinical → Operations: Care models require execution infrastructure · *REQUIRES*

A care model is a diagram until it has workforce, credentialing, administrative infrastructure, and operational management. The Collaborative Care Model requires a behavioral health care manager employed by a primary care practice. The CCBHC model requires a certified entity with the organizational capacity to serve all patients regardless of insurance status. Blueprint PCMHs require care coordinators, HIE connectivity, and NCQA recognition processes. None of these are available without Operations pillar investment.

VERMONT EVIDENCE

Oliver Wyman's Act 167 analysis found that Vermont's administrative cost per adjusted discharge runs far above the national benchmark — UVMHC at \$3,826 against a \$1,427 benchmark. This is not merely an inefficiency problem — it is a signal that Vermont's Operations pillar infrastructure is absorbing cost without producing the execution capability that clinical transformation requires. The RHRC engagement with all 14 Vermont hospitals is specifically designed to build the Operations pillar capacity that Clinical pillar programs need.

The Equity Imperative on Clinical — effective, and effective for whom?

Geographic and demographic disparities in access are primarily clinical access problems — not enough primary care providers, not enough behavioral health capacity, not enough community health workers in underserved communities — so the access-gap goals cannot be reached without the Clinical pillar's care-model expansion. Blueprint's PCMH expansion into Northeast Kingdom communities, the CCBHC network's geographic reach and CHT deployment in high-SDOH communities are Clinical investments that directly serve them. Access is not the whole test, though. Implicit bias, language barriers and differential treatment quality by race and ethnicity are clinical practice problems, so care models have to address cultural competence, language access and practice variation as design requirements, not implementation afterthoughts.

VERMONT EVIDENCE

Essex County's elevated uninsurance rate and limited primary care access are equity problems with a clinical solution — expanding Blueprint-participating practices in underserved HSAs, deploying telehealth through the RHT Program to reduce geographic access barriers, and certifying additional CCBHCs in Northeast Kingdom communities. Remove the Clinical pillar investment and the equity imperative's access gap remains unmeasured and unaddressed.

1.4.5 Operations Dependencies: Two Feedback Relationships That Close the Loop

Operations is the pillar that executes all other pillars' intentions. Its two outbound dependencies run back to Technology and Policy — feedback relationships that make the system self-correcting rather than purely top-down.

Operations → Policy: Implementation data feeds back into policy design · *INFORMS* ∪

This is the feedback loop that allows the policy architecture to be self-correcting. AHS's monthly transformation reports to the Vermont Legislature — required under Act 68 — document whether transformation is proceeding on the statutory timeline, where gaps have emerged, and what adjustments are

required. The GMCB’s RBP methodology for FY2027 is being refined based on hospital financial data from the operations layer. The December 2028 Statewide Strategic Plan will be drafted based on what the Operations pillar has learned from implementing the preceding three years of transformation. This is not passive reporting — it is active policy learning.

Operations → Technology: Workforce runs the data infrastructure · RUNS ∪

The Technology pillar’s infrastructure — VHCURES analytics, the CIN data platform, FHIR compliance, AI governance programs — does not operate itself. It requires IT staff to maintain systems, data analysts to generate the reports that clinical and financial management teams use, privacy officers to manage HIPAA compliance, and cybersecurity staff to protect the infrastructure from the ransomware threats that have disabled rural health systems nationally. The Operations pillar’s workforce is the human substrate on which the Technology pillar’s data infrastructure runs.

VERMONT EVIDENCE

Vermont’s rural CAHs — Grace Cottage, Gifford Medical Center, North Country Hospital — have limited IT staff. This Operations pillar constraint directly limits their Technology pillar capability. The Vermont CIN’s shared services model addresses this: a shared analytics and IT function within the CIN provides all 14 hospitals access to Technology pillar capability they cannot individually afford. The CIN is an Operations pillar solution to a Technology pillar gap.

The Equity Imperative on Operations — executable everywhere, including the rural and under-resourced?

The test for Operations is whether the capacity exists where the gaps are. Regionalization concentrates specialty care at regional centers; left unmanaged, it moves care farther from the communities that are already the most distant from a regional hospital and the most underserved by providers, and a plan that is executable in Burlington is not thereby executable in the Northeast Kingdom.

VERMONT EVIDENCE

Chapter 10 (§10.4.1) makes the Northeast Kingdom — Caledonia, Essex and Orleans counties — Vermont’s clearest equity priority, and argues that AHS must say so in the Statewide Strategic Plan: the priority constrains the pace and structure of regionalization, because the financial logic of consolidating services at regional hospitals in Burlington, Rutland or Bennington cannot override the obligation to keep emergency access in the state’s most isolated communities.

1.5 The Failure Cascade: What Happens When a Pillar Is Missing

5	9	5	1
Pillars, All Required	Dependency Relationships	Failure Cascade Patterns	Pillar Missing = System Fails

The most rigorous test of a dependency framework is not whether it describes how things work when all components are present — it is whether it accurately predicts what fails when a component is removed. The nine dependencies described above generate specific, testable failure-cascade predictions. The following analysis applies this test to each pillar.

Pillar removed	Immediate failure	Second-order failure	System-level outcome	Historical example
Policy	Payment reform remains voluntary; highest-cost actors opt out; rate benchmarks have no enforcement.	Clinical programs continue without the financial incentive alignment that makes population health investment rational; technology is built but not used for financial management.	Incremental improvement among willing participants; no system-level change; ongoing premium inflation; hospital financial crisis continues.	Vermont 2010–2022: OneCare voluntary ACO. Modest results; 9/14 hospitals in losses by 2023.
Technology	VBC contracts signed but not managed; attribution errors undetected; TCOC measurement unreliable; quality measures cannot be stratified.	Economics contracts produce unexpected losses; Clinical cannot identify high-risk patients before they hospitalize.	VBC produces losses for providers who cannot manage it; organizations exit VBC; payment reform stalls; equity gaps widen invisibly.	Most early-CMMI ACO failures: organizations entered models without analytics and produced unexpected losses.
Economics	Clinical programs continue under fee-for-service incentives; hospitals remain rewarded for volume, not value.	Primary care investment produces outcomes but cannot capture the financial return — the 5.8:1 ROI accrues to payers, not providers.	Clinical quality improves at the margin; costs keep rising; the financial case for transformation cannot be made; fragility deepens.	U.S. national experience: 20+ years of quality improvement without payment reform. Quality rose; cost growth continued.
Clinical	Payment reform changes incentives but has nothing to incentivize; global budgets create pressure without the programs that reduce cost.	Operations manages financial pressure through service cuts rather than care redesign.	Hospital financial crisis accelerates under global-budget pressure; rural hospitals close.	Maryland HSCRC early period: global budgets without adequate primary care produced pressure without sustainable improvement.
Operations	Statutory mandates exist on paper; clinical programs are designed; data platforms are procured; nothing is implemented.	Policy accountability fails without capacity to meet deadlines; platforms deployed but unused for management; contracts signed but not managed.	Transformation documents accumulate; deadlines slip; the 2028 Strategic Plan is descriptive rather than committal; the deficit trajectory continues.	Most state reform efforts of the 2010s: correct policy, adequate financing, sophisticated data — capacity insufficient to execute.

Figure 1.4 — Failure-cascade analysis: what breaks when each pillar is absent. Sources: HTR analysis; CMMI evaluation literature; Oliver Wyman Act 167 Report; Vermont experience.

1.6 Using the Dependency Map as an Analytical Tool

The dependency map is not merely a theoretical framework — it is a practical analytical tool for three management functions: transformation sequencing, risk identification, and investment prioritization.

1. Transformation sequencing — what must come first. The dependency structure generates a partial ordering of interventions: some investments cannot produce their expected results until enabling investments are in place. Getting the sequence wrong is not merely inefficient; §1.14 sets out what it costs. Sections 1.8 through 1.14 develop that ordering in full, returning to the OneCare failure examined in Section 1.2.

2. Risk identification — where a delay costs the most. A pillar with many direct dependents in Figure 1.3 carries outsized risk if it stalls, because the failure does not stay contained to that pillar. Technology is the clearest case: Economics and Clinical both depend on it directly, and right now it is where the system's most consequential execution gap actually sits. The AHS-GMCMCB analytics capability is the single highest-risk gap in the system: a delay there does not delay one program but three, holding up financial management for global budgets, equity measurement for the Statewide Strategic Plan, and risk stratification for Blueprint's CCBHC expansion at once.

3. Investment prioritization — where a dollar buys the most. The same logic identifies the highest-leverage investments: strengthening a pillar with many direct dependents pays off across all of them at once. That is why analytics deployment, AHS organizational capacity, and HCC gap closure are the highest-leverage investments available to Vermont right now.

1.7 The Vermont Thread: Five Pillars in One System

Vermont's healthcare transformation is the closest thing to a controlled experiment in simultaneous five-pillar intervention that American health policy has produced. The following table maps Vermont's primary reform vehicles onto the five-pillar framework, establishing the connective tissue that runs through every subsequent chapter.

Pillar	Primary Vermont intervention	Secondary interventions	Developed in
Policy	Acts 167 (2022), 51 (2023), 68 (2025): the legislative reform cascade; AHEAD Model State Agreement (January 2025; withdrawn July 2026)	Act 55 (drug pricing); Act 49 (insurer solvency); Act 62 (HIE governance); federal RHT Program award	Chapters 2–3
Technology	VHCURES all-payer claims database; Vermont CIN (RHT Program); AHS-GMCMCB data integration and analytics capability; AI scribe and RPM grants	VITL/VHIE HIE; DVHA HIT Plan (Act 62); statewide EHR feasibility assessment; FHIR compliance investments	Chapters 4–5
Economics	Reference-based pricing (mandatory, FY2027); global hospital budgets (FY2028–2030)	Act 68 hospital spending reduction (2.5%, FY2026); GMCMCB FY27 budget guidance (–1% commercial rate); Act 55 drug cap (\$135M savings)	Chapters 6–7
Clinical	Blueprint for Health (all-payer PCMH + CHT); CoCM and MHI behavioral health integration; CCBHC expansion (5 entities by July 2026)	Hub-and-Spoke SUD model; PACE program development; COE regionalization; dementia care infrastructure	Chapters 8–9

Pillar	Primary Vermont intervention	Secondary interventions	Developed in
Operations	14-hospital transformation planning (RHRC + AHS); Vermont CIN shared services; Act 68 transformation grants (\$2M); PMO establishment	EMS regionalization; administrative simplification (CON, prior auth); AHS restructuring; Division of Planning and Effectiveness	Chapters 11, 16

Figure 1.5 — Vermont's five-pillar transformation map. Sources: Acts 167, 51, 55, 62, 68 of 2022–2025; AHEAD State Agreement; Vermont RHT Program Application; Oliver Wyman Act 167 Report.

This table is a navigation map rather than a comprehensive inventory of Vermont's healthcare reform — a way of locating Vermont's specific interventions within the analytical framework that the subsequent chapters develop. The framework gives every Vermont policy intervention a structural home and every analytical concept a Vermont application.

1.8 From Framework to Sequence: Why Order Is Not Optional

Sections 1.3 through 1.7 established what the five pillars are and how they depend on one another. The dependencies establish something sharper still: that the pillars cannot be built in any order. A dependency is, by definition, a constraint on sequence — if Economics requires Technology, then Technology must come first.

Figure 1.3's seven build-order edges — the ones without a $\supset$ — form a strict precondition chain, and only one sequence satisfies all of them at once. Policy has no precondition, so it can move first. Technology's only precondition is Policy, so it can move second. Economics needs both Policy and Technology, already placed, so it can move third. Clinical needs Technology and Economics, already placed, so it can move fourth. Operations needs Policy and Clinical, both already placed, so it moves last. Policy, Technology, Economics, Clinical, Operations is not one reasonable order among several — it is the only order the matrix permits.

The rest of this chapter develops what that constraint costs when it is violated, using Vermont's OneCare failure as the case that proves it.

1.9 The Core Argument: Dependency Logic Determines Sequence

Strip away the state names and the claim is this: any organization or system attempting to move all five pillars — Policy, Technology, Economics, Clinical, and Operations — will fail if it deploys them against their dependency order, and the failure will be structural rather than a matter of effort, funding, or political will. This is true whether the actor is a state government, a health system, or a payer, and regardless of which pillar feels most urgent at the time. Vermont is the case where both halves of this claim — the failure when the order is wrong, and the correction once the order is fixed — are fully documented and publicly visible, which is why it carries the argument in this chapter.

Vermont did not choose careful sequencing because it was disciplined. It was forced into it because it had the scars from earlier experiences and had run out of time to get it wrong.

When Oliver Wyman presented Vermont's hospital financial trajectory in September 2024 — nine of fourteen hospitals in operating losses, thirteen of fourteen projecting losses by 2028, a cumulative five-year system deficit between \$700 million and \$2.4 billion — the room understood something that policy documents rarely

state directly: the cost of getting the sequencing wrong was now larger than the state's ability to recover. A decade of OneCare Vermont had demonstrated what the wrong sequence produces. Acts 167, 51, and 68 are the statutory attempt to build the right one.

The pillars are usually presented as simultaneous imperatives — each necessary, each dependent on the others. That is true, and it is the reason the framework holds. But it is useless to someone who has to commit a budget on Monday, because it says nothing about what to fund first. The answer is not arbitrary, and it is not a matter of institutional preference or political opportunity. .

The execution sequence is determined by dependency logic — the structural reality that some things cannot produce value until other things are already in place.

Getting the order wrong does not merely slow progress. It produces structural failures that are expensive to reverse, politically difficult to explain, and — as OneCare's experience shows — sometimes impossible to correct from within the organization that made the error.

The correct sequence is: Policy → Technology → Economics → Clinical → Operations. The critical and counterintuitive placement is Technology before Economics. Most reform architects reverse this, for understandable reasons: payment reform feels like the lever, technology feels like the support function. OneCare's failure is proof that this instinct is wrong.

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The Policy → Technology → Economics → Clinical → Operations sequence does not depend on a state having an all-payer model, a Green Mountain Care Board, or a single dominant ACO. Substitute any state's reform vehicle for OneCare and the same diagnostic question applies: did payment reform get layered onto a data infrastructure that did not yet exist? If so, the same cascade — unmeasurable risk, unmanageable contracts, eventual collapse — is the predictable result, not a Vermont-specific accident.

1.10 The Critics: Why Sequencing Logic Has Real Opponents

The sequencing argument presented in this chapter — and the OneCare autopsy in Section 1.2 that motivates it — has critics, who deserve direct engagement rather than dismissal.

1.10.1 “Sequencing Is a Post-Hoc Rationalization”

The most sophisticated critique: sequencing frameworks are analytical stories told after the fact to explain failures that had multiple causes. OneCare failed for many reasons — not just sequencing errors, but inadequate provider engagement, the complexity of Vermont's payer landscape, and the inherent difficulty of achieving savings under a CMMI model with multiple confounders. Calling it a “sequencing failure” imposes a cleaner narrative than the evidence warrants.

This critique has merit. OneCare's failure was overdetermined — it had more causes than sequencing alone. The sequencing analysis is useful not because it identifies the single cause but because it identifies the structural errors that made the failure architecturally inevitable regardless of operational quality. A car without brakes will crash regardless of how skilled the driver is. The sequencing failures in OneCare's design were the missing brakes.

1.10.2 “Mandatory Architecture Creates Provider Opposition That Delays Reform”

A second serious critique: by making participation mandatory, Vermont’s approach creates an adversarial relationship with hospital systems that could be productive reform partners under voluntary models. Hospital opposition to mandatory reference-based pricing has been fierce and legally aggressive. UVMHC challenged GMCB budget enforcement in court. A voluntary approach would face less resistance, move faster initially, and produce more durable buy-in.

The counterevidence is OneCare itself: a decade of voluntary reform produced modest results and left the financial crisis unaddressed.

No one is choosing between a smooth voluntary process and a contentious mandatory one. The real choice is between a process that produces structural change — contentiously — and one that produces durable accommodation of the status quo, smoothly.

Vermont’s legislature chose structural change. Chapter 14 (Political Sustainability) analyzes what it takes to maintain that choice across election cycles.

1.10.3 “Technology-First Is a Recipe for Expensive Shelfware”

A third critique targets the Technology-before-Economics sequencing specifically: building analytics infrastructure before the payment model that will use it often produces sophisticated technology that no one has the financial incentive or organizational capacity to deploy. Vermont’s VITL health information exchange is cited in this chapter as an example of technology deployed without the surrounding incentive structure — exactly the outcome the technology-first argument is supposed to prevent.

This critique identifies a real failure mode. The response is a distinction the framework makes explicitly: technology deployment must be coordinated with payment architecture design. Building VHCURES analytics capability in 2025–2026 while designing the Act 68 global budget methodology in parallel — so that the technology is built to serve a specific and known payment model — is different from building technology without a defined use case. The failure mode to avoid is not “technology before economics” but “technology without economics.”

1.11 The Five Stages: Resolved

1.11.1 Stage 1: Policy — Mandate and Capital

Policy is the absolute precondition for everything that follows. Its two functions are: create the mandatory architecture that prevents high-cost actors from opting out of reform, and secure the transformation capital that funds the technology build.

Vermont’s legislative progression illustrates both. Act 167 established the diagnostic foundation. Act 51 moved from diagnosis to planning, authorizing AHS to run transformation engagements with individual hospitals. Act 68 operationalized the mandate — converting voluntary reform into binding statutory requirements for global budgets and reference-based pricing. The AHEAD Model federal agreement was to add federal funding for the technology build; Vermont withdrew from it in July 2026, as the Introduction explains. The Rural Health Transformation award — \$195 million for Vermont in its first year (federal fiscal 2026) — is a Policy-pillar outcome, not an Economics-pillar outcome. Transformation capital comes from policy and regulatory relationships, not from the payment model itself.

1.11.2 Stage 2: Technology — Build the Substrate Before Assuming Risk

Technology is the operating system on which every subsequent stage runs. It is not a layer added on top of a functioning reform. It is the foundation that makes reform functional. The required components: integrated clinical and claims data; data updated in near-real time; and shared infrastructure for organizations that cannot independently support sophisticated analytics.

Vermont's VHCURES provides longitudinal population data. VITL/VHIE provides the clinical exchange. The AHS-GMCB analytics capability provides the modeling for global budget design and equity monitoring. The Vermont CIN provides shared capability to rural hospitals that individually lack the scale to build these functions.

The payment architecture design — Economics-as-design — happens in parallel during Stage 2. This is the work of specifying what the payment model needs to measure, which determines what the technology needs to do. But Economics-as-management — the operational work of running global budgets — does not go live until the technology substrate is functional.

1.11.3 Stage 3: Economics — Incentive Architecture on a Visible System

Economics is third because its management function requires the technology substrate to already be operational. Vermont's economic architecture has two structural components. Reference-based pricing, targeted at 200% of Medicare rates or below, addresses the unit price problem: Vermont's commercial insurance rates run far above Medicare, creating a cross-subsidy that is neither sustainable nor equitable. Global hospital budgets address the volume dimension: setting a fixed total revenue envelope for each hospital makes volume expansion financially irrational.

Together, reference-based pricing and global budgets break the fee-for-service trap that makes population health management economically punishing for providers. This is the distinction between Incentive Architecture and Transformation Capital in practice: global budgets and reference-based pricing permanently restructure the payment environment, while transformation capital — the money to build the workforce, telehealth infrastructure, and community health worker programs the transformed system requires — comes from the five-year, \$195-million-per-year Rural Health Transformation Program. Because that capital is time-limited, Vermont's standing source for community health investment beyond the RHT period is not yet settled.

The equity constraint becomes operationally urgent at Stage 3. Global budgets designed without social risk adjustment inadvertently penalize safety-net providers serving high-SDOH populations. Vermont's Northeast Kingdom is the clearest example: a global budget benchmark without social risk adjustment generates financial pressure that reflects social disadvantage, not operational inefficiency. Social risk adjustment must be embedded in the payment design, not retrofitted afterward.

1.11.4 Stage 4: Clinical — Redesigning Care on Aligned Incentives

Clinical redesign is where the economic return on the entire investment is realized. Patient-Centered Medical Homes transform primary care from acute episodic treatment into proactive population management. The Collaborative Care Model integrates behavioral health into primary care. Vermont's Blueprint for Health generated a 5.8-to-1 return on investment from its primary care investment — among the most robust ROI findings in the value-based care literature.

Clinical redesign comes fourth because clinical transformation without economic alignment reverts when the incentives revert. Clinicians redesign their practice in response to the incentives they face and the data available to them. Without a payment model that rewards the redesigned care — already live at Stage 3 — and without the technology to identify which patients need which interventions — already built at Stage 2 — clinical

transformation is isolated, unsustainable, and unable to demonstrate the population-level outcomes that justify its cost.

1.11.5 Stage 5: Operations — Closing the Administrative Cost Gap

Operations is the execution layer that converts all prior-stage investments into organizational reality. Vermont's PPS hospitals spend \$2,730 per adjusted discharge on management and administration, compared with a national benchmark of \$1,427 — and UVMHC alone runs \$3,826 against that same benchmark. This \$1,303 per-discharge gap across the PPS group represents the primary source of the \$100 million or more in projected direct savings from shared services and administrative simplification.

Operations is last not because it is least important, but because operational redesign requires the technology infrastructure, payment incentives, and clinical models of the preceding stages to already be in place.

A hospital that redesigns its administrative operations before its payment model is live will optimize for a fee-for-service world that the reform is designed to replace.

Summary Table of the Five Stages and Their Execution Sequence:

#	Pillar	Key action	Why this sequence	Vermont anchor
1	Policy	Establish enforceable authority, mandate participation, secure transformation capital	Nothing can be authorized, built, or funded without statutory permission. Mandatory architecture prevents high-cost actors from opting out.	Acts 167, 51, 68; \$195M first-year RHT award
2	Technology	Build integrated clinical + claims data infrastructure — real-time — before financial risk is assumed	Technology is the operating system. Must be operational before Economics goes live, for the reasons the OneCare autopsy sets out above.	VHCURES; VITL/VHIE; AHS-GMCH analytics capability; Vermont CIN
3	Economics	Deploy global budgets and RBP onto a system that can already see itself	Economics-as-management requires the technology substrate. Payment design (Economics-as-design) happens in parallel during Stage 2.	Global hospital budgets; RBP at ≤200% Medicare; EAST Fund (roughly \$138M expected; cut to a cap near \$10M when Vermont withdrew from AHEAD)
4	Clinical	Redesign care on aligned incentives	Clinical transformation is where ROI is realized — but only when payment incentives are live and technology identifies which patients need which interventions.	PCMH transformation; CoCM; CCBHC expansion; Blueprint 5.8:1 ROI
5	Operations	Close the administrative cost gap and translate strategy into implemented programs	Operations is the execution layer. Vermont's \$1,303 per-discharge administrative gap vs. national benchmark is the primary source of projected direct savings.	RHRC methodology; shared services; 14-hospital transformation planning

Figure 1.6 — The Five-Stage Execution Sequence. Source: HTR Analysis (2026).

1.11.6 The Equity Imperative: A Constraint Across Every Stage

Equity is not a stage in the sequence — it is the Equity Imperative applied across every stage, a constraint that runs through each rather than a final add-on applied to a finished system. Vermont's 11-point primary care access gap between white and BIPOC residents is not closed by a late audit. It is closed by the stages working in sequence with equity built in: payment design that does not penalize safety-net providers (Economics), clinical redesign deploying Community Health Teams in underserved areas (Clinical), and the measurement that checks whether those interventions are actually closing the gap.

What the Equity Imperative adds is systematic, population-level measurement of whether the equity commitments embedded in every stage have produced equitable outcomes — and the corrective action, informed by real data from a functioning system, when they have not.

1.12 Three Sequencing Principles in Practice

Correctly applying the five-stage sequence to real investment decisions requires three operating principles — the rules a reform architect uses to decide what to fund now, what to fund in parallel, and what to hold until an upstream gate opens.

1.12.1 Principle 1: The Critical Path Is Non-Negotiable

The five-pillar framework identifies three critical-path dependencies: Policy enables Economics, Economics drives Clinical, and Technology enables Economics-as-management. These are not preferences or best practices — they are structural requirements. Violating them does not produce suboptimal results; it produces failed interventions that generate political resistance to the correct interventions that follow.

Critical-path dependencies cannot be shortcut by allocating more resources to the downstream intervention. A hospital that signs a full-risk global budget contract before its VHCURES analytics capability is operational has not accelerated its transformation — it has exposed itself to financial losses it cannot explain, cannot dispute, and cannot manage. The analytics capability cannot be retroactively built while the contract is running. The sequence cannot be reversed after it has been violated.

In practice, reading the critical path means identifying which investments are *gates*.

Vermont's critical-path sequence for hospital global-budget implementation under Act 68 reads as four gates in order:

- **The Policy gate — OPEN since June 2025.** Act 68 established mandatory RBP (FY2027) and global budgets (FY2028). All Vermont hospitals are now inside the mandatory architecture, so nothing downstream is waiting on statutory authority.
- **The Technology gate — PARTIALLY OPEN; the current bottleneck.** VHCURES analytics, HCC gap closure and attribution modeling must all be operational before Act 68's hospital global budgets take effect in FY2028.
- **The Economics gate — CLOSED until Technology opens.** The global budget contracts GMCB sets for FY2028 cannot be managed before the analytics that measure performance under them exist.
- **The Clinical gate — CLOSED until clinical programs reach scale.** Utilization reduction must be operating before the full financial accountability of global budgets takes effect.

1.12.2 Principle 2: Parallel Work Is Possible — But Not on Critical-Path Dependencies

The critical path is sequential by definition. But the framework does not require that *all* work be sequential. Many investments can proceed in parallel, as long as none of the parallel tracks violates a critical-path dependency.

Vermont’s current architecture illustrates this. While the Technology gate is still being opened — analytics capability deployment, VHCURES integration — work proceeds in parallel on Equity design (social risk adjustment methodology for global budgets), Operations capacity building (RHRC hospital transformation planning, AHS PMO establishment), and Clinical infrastructure (Blueprint PCMH expansion, CCBHC certifications).

None of that parallel work sits behind the Technology gate. Designing a social risk adjustment methodology, training hospital transformation teams and certifying CCBHCs all produce their full intended value whether or not the analytics are running yet, so they can proceed now without violating any critical-path dependency.

The diagnostic question for any proposed parallel investment: *does this investment have an unmet upstream dependency?* If yes — if its effectiveness depends on a gate that is not yet open — it is not parallel work. It is premature work that will produce either wasted results or results that cannot be used until the upstream dependency is met.

1.12.3 Principle 3: Equity Is a Design Constraint, Not a Downstream Filter

The most common sequencing error organizations make with the equity imperative is treating equity review as the last step before a decision is finalized rather than as a constraint embedded in the design process.

Equity-as-last-step produces expensive redesigns at the approval stage. Equity-as-design-constraint asks the equity-implication question at the *beginning* of the design process — shaping the instrument rather than filtering it.

In practice, embedding equity as a design constraint requires two organizational changes: adding equity review to the *initiation* criteria for policy and program design (not only the approval criteria), and ensuring that Health Equity Advisory Commission consultation under Act 68 happens early in the design cycle rather than at the comment stage. Vermont’s statutory framework requires equity consultation — it does not specify when in the process that consultation must occur. Organizations that treat it as early design input rather than late validation produce more equitable outcomes with less redesign cost.

TRY THIS — Test a critical-path violation.

Open the **HTR Simulator** and model an organization that funds a downstream pillar before its upstream gate is open — for example, high Economics ambition with a weak Technology score. Watch the composite readiness collapse. Then open the **Transformation Friction Index** to see which pillar is your binding constraint. The principle is simpler to feel in the tool than to argue on the page: no amount of downstream funding opens an upstream gate. Spending more on the payment model does not make the missing analytics appear.

GO DEEPER — ACADEMY

The dependency mechanics behind these three principles are the subject of the Academy’s **Five Pillars, One Imperative** course — specifically Lesson 4, “Dependency Logic and the Execution Sequence,” and Lesson 5, “Sequencing Failure — The OneCare Autopsy.” Start there before applying the sequencing framework to your own organization or state.

1.13 Five Sequencing Decisions That Organizations Get Wrong

The OneCare autopsy in Section 1.2 is the fully documented case. The same errors recur, in generalizable form, across every transformation effort. **Figure 1.7** is the diagnostic version — the pattern to recognize, the early-warning signals that reveal it before it fully cascades, and the correction — so a reform architect can catch a sequencing error in their own system while it is still cheap to reverse.

Sequencing error	Early-warning signals	Vermont evidence	Correction
Economics without Policy readiness — payment reform launched without mandatory participation	Voluntary participation below 80%; highest-cost payers or providers stay out; redesigns driven by adverse selection	OneCare Vermont (voluntary architecture normalized over roughly a decade, making the later transition to mandatory reform more disruptive than it would have been at the outset)	Act 68 mandatory RBP + global budgets
Technology without Operations readiness — platform deployed to an org that cannot use it	Adoption below 30% six months post-deployment; continued manual processes; staff workarounds	VITL: sound platform, incomplete adoption among smaller providers for lack of Operations-pillar support to connect and train them	Vermont CIN shared-services model
Clinical without financial alignment — high-quality program run under fee-for-service	Program running at a loss absorbed by cross-subsidy; support contingent on that cross-subsidy; perpetual budget-cut risk	Blueprint PCMHs produce the 5.8:1 ROI, but under FFS most of it accrues to payers, not the practices generating it	Act 68 global budgets align incentives with clinical investment
Equity review at the end, not the beginning — methodology finished, then sent for equity review	Equity staff flagging the same structural problems in finalized designs; high rate of equity modifications at approval stage	RBP methodology design is the live test case: preserving rural access and not defunding high-Medicaid hospitals is a design question, not a post-hoc correction	Health Equity Advisory Commission consulted at design stage, not comment stage
Operations execution without adequate capacity — mandate outpaces organizational capacity	Statutory deadlines extended or waived; deliverables perpetually “in progress”; governance that convenes but does not decide	AHS must run 14 hospital transformation processes, RBP finalization, and the Strategic Plan at once — capacity Oliver Wyman flagged as insufficient	RHRC builds hospital-level capacity before global-budget accountability takes full effect

Figure 1.7 — Five sequencing errors: the diagnostic pattern, early-warning signals, and correction for each. Sources: HTR analysis; Vermont experience; Oliver Wyman Act 167 Report.

1.14 Vermont’s Implementation Timeline: Reading the Sequence in Statutory Deadlines

Vermont’s transformation is unusual in one critical respect: it has a legislatively mandated implementation timeline that is legally binding, not aspirationally stated. Reading this timeline through the sequencing framework reveals both Vermont’s correct sequencing decisions and its current critical vulnerabilities.

Pillar	Status (timeframe)	Where it stands
Policy	Completed (2022–2025)	Acts 167, 51, and 68 establish the mandatory architecture. Federal RHT award provides capital. AHEAD State Agreement signed January 2025 (subsequently withdrawn July 2026). The Policy gate is fully open — the statutory architecture is complete, so nothing downstream is waiting on it.
Technology	In progress (2025–2026)	AHS-GMCB analytics capability being stood up. Vermont CIN under development. VHCURES attribution modeling in progress. AI scribe and RPM grants deployed. The current critical bottleneck. The Technology gate is partially open: some capability is running, but not enough for the Economics gate behind it to open.
Economics	Staged (2027–2028)	RBP mandatory beginning FY2027, global budgets mandatory beginning FY2028. The sequence is correct: mandatory payment reform follows mandatory policy authority and follows the technology build in design.
Clinical	Parallel (2024–2028)	Blueprint PCMH expansion, CCBHC certifications, CoCM deployment proceed in parallel with Technology work — correctly, because they produce their value regardless of whether the Technology and Economics gates have opened yet.
Operations	Building (2025–2028)	RHRC engagement, AHS HSA Coordinator model, Division of Planning and Effectiveness, PMO structure — correctly sequenced to precede the peak execution demand of FY2027–2028.

Figure 1.8 — Vermont’s transformation timeline, by pillar. Sources: HTR Analysis (2026); Act 68 of 2025; AHEAD State Agreement.

1.14.1 The Current Vulnerability: The Technology-Economics Race

Vermont’s most significant current sequencing risk is the gap between the FY2028 start of Act 68’s hospital global budgets and the analytics-capability deployment timeline. Vermont hospitals will begin bearing global-budget financial accountability in January 2028. If the AHS-GMCB analytics capability is not fully operational by that date, hospitals will be making financial management decisions based on incomplete or lagged data for the first year of full Act 68 global-budget accountability.

This is a manageable but costly risk. Vermont hospitals that invest in their own VHCURES analytics capability — particularly HCC gap closure, the highest-ROI pre-launch analytics investment — can partially compensate for the analytics capability gap. A delay in that capability’s deployment does not merely delay one program: it delays the financial management capability for Act 68 global budgets, the equity measurement capability for the Statewide Strategic Plan, and the risk stratification capability for Blueprint’s CCBHC expansion. Three dependencies converge on this single Technology pillar investment.

1.15 EXPLORE ON THE PLATFORM

The execution-sequence argument is best understood by manipulating it, not just reading it. The HTR platform turns this chapter’s three core claims — that the pillars are interdependent, that order is non-negotiable, and that getting the order wrong produces a measurable failure cascade — into tools you can run against your own organization or state.

Do this	On this tool	What to look for
Trace the 9 dependency relationships between the five pillars and see which downstream pillars a given gap disables	Five-Pillar Map — /about/framework	Click any pillar to highlight what it enables and what it depends on. Confirm visually why Technology sits upstream of Economics.
Score a real organization or state across all five pillars and watch the composite readiness number respond	HTR Simulator	Drop one pillar's score to zero and observe the cascade — the simulator penalizes downstream readiness, reproducing the OneCare logic in Fig 1.1.
Quantify where reform is losing momentum and which pillar is the binding constraint	Transformation Friction Index	Identify your own “Technology gate” bottleneck — the chapter's central Vermont vulnerability — before it becomes a financial one.
Model how one pillar's shortfall propagates through the other four over time	Impact Simulation — /impact-simulation	The lag: a Technology gap shows up in Economics results a year later, which is why it is missed.

Figure 1.9 — Hands-on platform tools for Chapter 1: working the execution-sequence argument on the HTR platform.

TRY THIS — Reproduce the OneCare cascade.

Open the **HTR Simulator** and build a profile with strong Economics ambition (high) but weak Policy mandate (voluntary) and weak Technology substrate (no integrated real-time data). The composite score should collapse — not because the Economics inputs are bad, but because the simulator enforces the same dependency logic that sank OneCare. Then fix the order: raise Policy and Technology first, and watch the same Economics inputs finally produce a viable score. That reversal *is* the argument of this chapter.

1.16 GO DEEPER — ACADEMY

This chapter is the subject of Track 1, “Foundations: The Framework,” in the Academy’s **Five Pillars, One Imperative** course. Start at /academy — Track 1 is built to be worked alongside this chapter, not after it.

Lesson	What it covers
1 — Why Single-Pillar Thinking Fails	The CMMI record and why isolated interventions run into the constraints the other pillars create
2 — The Five Pillars and Their Diagnostic Questions	The five-question framework this chapter defines
3 — The Equity Imperative: Is It Just?	Why equity is a cross-cutting test, not a sixth pillar
4 — Dependency Logic and the Execution Sequence	The dependency matrix and the ENABLES/REQUIRES/DRIVES/∪ vocabulary
5 — Sequencing Failure — The OneCare Autopsy	The three-part failure this chapter opens with, worked in full

Figure 1.10 — Academy Track 1 lessons mapped to this chapter.

1.17 Implications for You

If you are a Vermont hospital executive: The most important action you can take before Act 68's global budgets take effect in FY2028 is completing HCC gap closure for your full Medicare-attributed population. This single Technology pillar investment improves risk adjustment accuracy, produces a more defensible global budget benchmark, and prevents the “Managing Blind” failure mode in your first Act 68 global budget performance year. Do not wait for the AHS-GMGB analytics capability to complete this work. Start with VHCURES direct access and your current analytics resources now.

If you are an AHS or GMGB official: The Technology gate is your critical bottleneck, and it has a hard deadline — analytics capability must be operational before Act 68's FY2028 global budgets take effect. The three-dependency convergence — global-budget management, Statewide Strategic Plan measurement, CCBHC risk stratification — means that a delay in the analytics capability is a compound failure across multiple statutory obligations. This deserves the urgency of a critical-path emergency, not the pace of a standard procurement.

If you are a Vermont legislator: The sequencing framework translates directly into the right oversight questions to ask AHS and GMGB at every reporting interval: Is the Technology gate open — analytics actually operating, not merely procured — before the Economics gate goes live? Is the RBP methodology being designed with equity constraints from the beginning, or will the equity review happen at the approval stage? Is the RHRC engagement building hospital-level Operations capacity ahead of Act 68's global budget accountability requirements? These three questions will determine whether Act 68's mandates produce the transformation they were designed for.

If you are a national health policy professional: Vermont's OneCare failure is the most precisely documented sequencing failure in recent American health policy. Before designing or evaluating any state total-cost-of-care model, ask three questions: Is participation mandatory or voluntary? Is the technology substrate operational before financial risk is assumed? Is the equity constraint embedded in the payment design or appended at the review stage? The answers predict outcome more reliably than any other design feature of the model.

1.18 Key Concepts in This Chapter

Dependency logic The principle that some pillar investments cannot produce results until upstream enabling investments are in place. Determines the execution sequence.

Execution sequence The order in which the five pillars must be built: Policy, Technology, Economics, Clinical, Operations. Determined by structural dependency, not by the relative importance of each pillar.

Critical path gate A prerequisite investment that must be in place before a downstream pillar investment can be productive. A gate is *open* when that work is finished and *closed* while it is unfinished; it is opened by completing the work, never by deciding to proceed. Cannot be bypassed by allocating more resources downstream.

“Managing Blind” failure mode Assuming downside financial risk under a value-based payment model without the analytics infrastructure to track total cost of care in real time. Produces unexpected, unexplainable losses.

Sequencing failure Deploying a downstream pillar before the upstream enabling investment is in place. OneCare Vermont is canonical: Economics (voluntary ACO) deployed without Policy mandate and Technology substrate.

Voluntary vs. mandatory architecture The distinction between reform programs actors can opt out of and those they cannot. Act 68 establishes mandatory RBP and global budgets — eliminating voluntary opt-out.

Economics-as-design vs. Economics-as-management Design (defining the payment architecture) can proceed in parallel with the technology build. Management (operating the model) requires the technology substrate to be operational first.

Transformation capital vs. incentive architecture Transformation capital (the RHT award, and the EAST Fund until Vermont's withdrawal from AHEAD) is time-limited bridge funding for infrastructure. Incentive architecture (global budgets, RBP) is permanent structural change to the payment environment. Confusing the two produces unsustainable programs.

Reform cascade A legislatively structured sequence where each intervention creates capacity and legitimacy for the next. Vermont's Act 167 → Act 51 → Act 68 is the prototype.

HCC gap closure The highest-ROI pre-global-budget analytics investment: completing hierarchical condition category documentation for all Medicare-attributed patients before global budgets take effect. Improves risk adjustment accuracy and produces a more defensible global budget benchmark.

Parallel work Investment that proceeds simultaneously with critical path work because it sits behind no closed gate. Test: could this investment produce its full intended value today, with upstream pillars in their current state?

Premature investment An investment in a downstream pillar that proceeds before the upstream enabling investment is in place. Most common forms: Technology deployment without Operations readiness; Economics contracts without Technology analytics readiness.

Sources: Vermont Agency of Human Services, Health Care System Transformation Report (November 2025); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations (August 2024, revised October 2024 and January 2025); Vermont AHEAD State Agreement (January 2025); CMS AHEAD Model documentation (September 2025 update); Vermont Blueprint for Health Annual Report 2024; Act 68 of 2025; OneCare Vermont ACO annual reports (2014–2023); Maryland HSCRC Total Cost of Care Model data; GMCB FY25 and FY26 Hospital Budget Orders; Gobeille v. Liberty Mutual Ins. Co., 577 U.S. 312 (2016); VTDigger, "Vermont's largest health insurer steps away from OneCare" (Dec. 20, 2022); VTDigger, "Vermont's 8-year 'all-payer' health care experiment sunset at the end of 2025. What did it teach us?" (Mar. 13, 2026); Vermont State Auditor performance audits of OneCare Vermont (2019, 2021).